

Admission

12/3/2025
SMRMC 4 NE TELEMETRY

NV Saint Mary's Regional Medical Group Galena
Location

Printout Information

Document Contents	Document Received Date	Document Source Organization
Encounter Summary	May 12, 2026	Prime Healthcare
Michelle Caruso - 71 y.o. Female; born Feb. 19, 1955		Encounter Summary, generated on May 12, 2026

Progress Notes - documented in this encounter

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[Tracy L Birkner, PA - 12/05/2025 8:48 AM PST](#)

[Manvi R Nagam, MD - 12/04/2025 11:24 AM PST](#)

Tracy L Birkner, PA - 12/05/2025 8:48 AM PST

Formatting of this note is different from the original.

CARDIOLOGY HOSPITAL FOLLOW-UP / PROGRESS NOTE

Tracy L Birkner, PA

Hospital Attending Manvi R Nagam, MD

Michelle Caruso

2/19/1955 (70 y.o.)

MRN: 116253192

Acct# 730000366346

History of Present Illness - Clinical Update

Michelle Caruso is a 70 y.o. female still works, PMH of mild CAD (30% LAD stenosis), hx of NSTEMI/MINOCA due to Takutsubo 7/2024 (OCT showed no SCAD, it was probably Takutsubo vs vasospasm vs microvascular dysfunction), moderate MR, and HFpEF, whom cardiology is consulted for NSTEMI.

Patient did not present with CP, she has a lot of stress at work, she presented with dizziness and SOB.

She has not been taking ASA or statin or BB/CCB/Nitro or Lasix and instead taking supplements.

Labs showed HsT ~3000.

12/5- very anxious about angiogram but overall feels well without CP or SOB

Past Medical, Family and Social History

Past Medical History:

Diagnosis Date

Broken heart syndrome

Interstitial cystitis

Past Surgical History:

Procedure Laterality Date

CYSTOSCOPY

DILATION AND CURETTAGE OF UTERUS

LAPAROSCOPY FOR ECTOPIC PREGNANCY

PR ARTHRP ACETBLR/PROX FEM PROSTC AGRFT/ALGRFT Right 3/4/2025

Procedure: RIGHT TOTAL HIP ARTHROPLASTY, DIRECT ANTERIOR APPROACH; Surgeon: Jessell M Owens, MD;

Location: NV SMRMC OR; Service: Orthopedics Surgery

SHOULDER SURGERY Right

Family History

Problem Relation Age of Onset

Cancer Father

Heart disease Father

Family Status

Relation Name Status

Father (Not Specified)

No partnership data on file

Social History

Socioeconomic History

Marital status: Single

Spouse name: Not on file

Number of children: Not on file

Years of education: Not on file

Highest education level: Not on file

Tobacco Use

Smoking status: Never

Smokeless tobacco: Never

Vaping Use

Vaping status: never used

Substance and Sexual Activity

Alcohol use: Yes

Comment: 5 per year

Drug use: Never

Sexual activity: Not on file

Medications Prior to Admission

Medication Sig Dispense Refill Last Dose

[EXPIRED] aspirin 81 MG EC tablet Take 1 tablet (81 mg total) by mouth daily

Medications and Allergies

Scheduled Meds:

aspirin 81 mg Oral Daily

atorvastatin 40 mg Oral HS

cefTRIAxone (ROCEPHIN) IVPB 1 g Intravenous Q24H

doxycycline 100 mg Intravenous Q12H

Pharmacy to dose Heparin 1 Consult Does not apply See Admin Instructions

Continuous Infusions:

heparin 12 Units/kg/hr 11 Units/kg/hr (12/05/25 0056)

PRN Meds: acetaminophen

artificial tears

bisacodyl

docusate sodium

heparin (porcine)

heparin (porcine)

heparin (porcine)

hydrALAZINE

nitroglycerin

ondansetron

ondansetron

polyethylene glycol

Allergies

Allergen Reactions

Citrus

Codeine GI Intolerance

Coffee Flavor [Flavoring Agent]

Gluten Meal

Vinegar [Acetic Acid]

Review of Systems (Complete=10 or more; Extended=2-9; Problem pertinent=1)

Review of Systems

Cardiovascular: Negative for chest pain.

Respiratory: Negative for cough and shortness of breath.

Musculoskeletal: Negative for back pain.

Gastrointestinal: Negative for abdominal pain, nausea and vomiting.

Psychiatric/Behavioral: Negative for altered mental status.

Physical Exam (Comprehensive 8; Detailed 5-7; Expanded Problem Focused 2-4; Focused 1)

24 Hour Min / Max Current

Temp Min: 97.8 °F (36.6 °C) Max: 98.6 °F (37 °C) 98 °F (36.7 °C)

BP Min: 102/62 Max: 137/78 132/78

Pulse Min: 63 Max: 66 63

Resp Min: 16 Max: 18 18

SpO2 Min: 94 % Max: 96 % 95 %

Admit: 76.6 kg (168 lb 14 oz) 80 kg (176 lb 5.9 oz) Body mass index is 28.47 kg/m².

I/O

12/03 0701

12/04 0700 12/04 0701

12/05 0700 12/05 0701

12/06 0700

P.O. 0 1320

I.V. (mL/kg) 174.2 (2.2)

Total Intake(mL/kg) 0 (0) 1494.2 (18.7)

Urine (mL/kg/hr) 0

Emesis/NG output 0

Stool 0

Total Output 0

Net 0 +1494.2

Unmeasured Urine Occurrence 2 x 8 x

Unmeasured Stool Occurrence 0 x 0 x

Physical Exam

Constitutional: She appears healthy. No distress.

Cardiovascular: Normal rate, regular rhythm, S1 normal, S2 normal and normal pulses.

No murmur heard.

Pulmonary/Chest: Effort normal and breath sounds normal. She has no wheezes. She has no rales.

Abdominal: Soft.

Musculoskeletal:

General: No edema.

Neurological: She is alert and oriented to person, place, and time.

Skin: Skin is warm and dry.

Data Review:

Results from last 7 days

Lab Units 12/04/25

0421 12/04/25
0724 12/05/25
0436 12/05/25
0437
WBC AUTO 10*3/uL 4.90 -- 4.60 --
HEMOGLOBIN g/dL 11.8* -- 12.2 --
HEMATOCRIT % 34.9 -- 35.5 --
PLATELETS AUTO 10*3/uL 223 -- 207 --
SODIUM mmol/L 141 -- -- 140
POTASSIUM mmol/L 3.6 -- -- 4.1
CHLORIDE mmol/L 106 -- -- 108
CO2 mmol/L 26.0 -- -- 25.0
BUN mg/dL 8.0 -- -- 7.0
CREATININE mg/dL 0.80 -- -- 0.70
GLUCOSE mg/dL 93 -- -- 97
CALCIUM UA mg/dL 9.2 -- -- 9.8
MAGNESIUM mg/dL 1.90 -- -- 1.90
PHOSPHORUS mg/dL -- -- -- 3.8
BILIRUBIN TOTAL mg/dL -- -- -- 0.5
AST U/L -- -- -- 22
ALT U/L -- -- -- 10
ALK PHOS U/L -- -- -- 64
ALBUMIN g/dL -- -- -- 4.1
HS TROPONIN I ng/L 1,366* 1,300* -- --
BNP pg/mL 75.47 -- -- --
CHOLESTEROL mg/dL 198 -- -- --
TRIGLYCERIDES mg/dL 48 -- -- --
HDL CHOLESTEROL mg/dL 74.0 -- -- --
TSH uIU/mL 1.342 -- -- --

Invalid input(s): "PCO2", "PO2", "HCO3", "O2SAT"

X-ray Chest 1 View

Result Date: 12/4/2025

: 1. No acute chest disease. This document has been electronically signed by: Andrew Ciccarelli, MD on 12/04/2025 02:44:51

TELE: sinus 60-70's

Cardiology Studies:

Echocardiogram 12/4/25:

Conclusion

1. The left ventricle is normal in size and systolic function with an ejection fraction of 60-65% by Simpson's biplane. There are no wall motion abnormalities noted. There is mild concentric left ventricular hypertrophy. There is grade 1 (impaired relaxation) diastolic dysfunction with normal LV filling pressures.
2. There are no significant valvular abnormalities.
3. The right ventricle is normal in size and function; RVSP is indeterminate.

**No prior echocardiogram for comparison.

Assessment and Plan

Assessment: NSTEMI (non-ST elevated myocardial infarction) (CMS/HCC)

1. NSTEMI
2. Hx of MINOCA 7/2024 (likely Takutsubo, but possibly spasm or microvascular dysfunction)
3. Mild LAD disease
4. Trace MR
5. HFpEF

Plan:

NPO for coronary angiogram today, currently agreeable to proceed however states she does not want a stent placed. Lengthy discussion had regarding treatment options of CAD depending on angiogram findings as well as antiplatelet therapy. Will have her discuss further with MD

Continue IV heparin/asa/statin

Tracy L Birkner, PA
Electronic Signature
12/5/2025 8:48 AM

Cosigned by Mohammad M Abu Sheikha, MD at 12/05/2025 12:09 PM PST

Electronically signed by Tracy L Birkner, PA at 12/05/2025 9:14 AM PST

Electronically signed by Mohammad M Abu Sheikha, MD at 12/05/2025 12:09 PM PST

Associated attestation - Abu Sheikha, Mohammad M, MD - 12/05/2025 12:09 PM PST

Formatting of this note might be different from the original.

Patient seen and examined. Patient's pertinent labs, EKG, and cardiac imaging/procedures details reviewed. I have personally met with the patient and discussed the plan of care:

70 y/o F still works, PMH of mild CAD (30% LAD stenosis), hx of NSTEMI/MINOCA due to Takutsubo 7/2024 (OCT showed no SCAD, it was probably Takutsubo vs vasospasm vs microvascular dysfunction), moderate MR, and HFpEF, whom cardiology is consulted for NSTEMI.

Patient did not present with CP, she has a lot of stress at work, she presented with dizziness and SOB.

She has not been taking ASA or statin or BB/CCB/Nitro or Lasix and instead taking supplements.

Labs showed HsT ~3000.

Plan:

1. NSTEMI
2. Hx of MINOCA 7/2024 (likely Takutsubo, but possibly spasm or microvascular dysfunction)
3. Mild LAD disease
4. Moderate MR with HFpEF

- TTE did not show Takutsubo.

- I do not think this is due to obstructive CAD given LHC findings from 7/2024, this is likely vasospasm or microvascular dysfunction. However, we will need to confirm. Continue heparin. Coronary angiogram today.

- Continue ASA 81 mg daily and Lipitor 40 mg daily.

- Further recs pending cath results.

Additionally, I agree with the plan of care as detailed in the note below.

Mohammad Abu Sheikha, MD FACC
Board Certified Cardiologist

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Manvi R Nagam, MD - 12/04/2025 11:24 AM PST

Formatting of this note is different from the original.
HOSPITALIST PROGRESS NOTE

Name: Michelle Caruso MRN: 116253192
DOB: 2/19/1955 Acct: 730000366346
Age: 70 y.o. Admit Date: 12/3/2025 7:58 PM
Sex: female Attending: Manvi R Nagam, MD
PCP: Sarah Maples Location: SMRMC 4 NE TELEMETRY

Length of Stay: 1

Date of service: 12/4/2025

Subjective

Patient denies any chest pain, shortness of breath or lightheadedness.

Echocardiogram pending.

Current Medications:

Scheduled Meds

aspirin 81 mg Oral Daily
cefTRIAxone (ROCEPHIN) IVPB 1 g Intravenous Q24H
doxycycline 100 mg Intravenous Q12H
Pharmacy to dose Heparin 1 Consult Does not apply See Admin Instructions

Infusing Meds

heparin 12 Units/kg/hr 11 Units/kg/hr (12/04/25 1620)

PRN Meds

acetaminophen, 650 mg, Q6H PRN
artificial tears, 1 drop, Q1H PRN
bisacodyl, 10 mg, Daily PRN
docusate sodium, 100 mg, Daily PRN
heparin (porcine), 25 Units/kg, Q6H PRN
heparin (porcine), 25 Units/kg, Q6H PRN
heparin (porcine), 50 Units/kg, Q6H PRN
hydrALAZINE, 10 mg, Q4H PRN
nitroglycerin, 0.4 mg, Q5 Min x3 PRN
ondansetron, 4 mg, Q4H PRN
ondansetron, 4 mg, Q6H PRN
polyethylene glycol, 17 g, Daily PRN

Objective

Vitals

Current 24 Hour Min / Max
Temp 98.6 °F (37 °C) Temp Min: 97.7 °F (36.5 °C) Max: 98.6 °F (37 °C)
BP 102/62 BP Min: 102/62 Max: 144/80
Pulse 66 Pulse Min: 59 Max: 68
Resp 16 Resp Min: 10 Max: 20
SpO2 94 % SpO2 Min: 93 % Max: 97 %

Weight 80.2 kg (176 lb 12.9 oz) Initial Weight 76.6 kg (168 lb 14 oz)
Body mass index is 28.54 kg/m².

Intake / Output I/O

12/03 0701
12/04 0700 12/04 0701
12/05 0700
P.O. 0 720
I.V. (mL/kg) 174.2 (2.2)
Total Intake(mL/kg) 0 (0) 894.2 (11.1)
Urine (mL/kg/hr) 0
Emesis/NG output 0
Stool 0
Total Output 0
Net 0 +894.2

Unmeasured Urine Occurrence 2 x 4 x
Unmeasured Stool Occurrence 0 x

Net IO Since Admission: 894.18 mL [12/04/25 2031]

Vent Settings

Physical Exam

Constitutional:

General: She is not in acute distress.

Appearance: Normal appearance.

HENT:

Head: Normocephalic.

Eyes:

Extraocular Movements: Extraocular movements intact.

Conjunctiva/sclera: Conjunctivae normal.

Cardiovascular:

Rate and Rhythm: Normal rate and regular rhythm.

Heart sounds: Normal heart sounds.

Pulmonary:

Effort: No respiratory distress.

Breath sounds: Normal breath sounds. No wheezing.

Abdominal:

General: There is no distension.

Palpations: Abdomen is soft.

Tenderness: There is no abdominal tenderness.

Musculoskeletal:

Cervical back: Normal range of motion.

Right lower leg: No edema.

Left lower leg: No edema.

Skin:

Findings: No lesion or rash.

Neurological:

General: No focal deficit present.

Mental Status: She is alert and oriented to person, place, and time.

Psychiatric:

Mood and Affect: Mood normal.
Behavior: Behavior normal.

Select Labs (last 7 days):
ABG

Anemia

CBC

Results from last 7 days

Lab Units 12/04/25

0421

WBC AUTO $10^3/\mu\text{L}$ 4.90

RBC AUTO $10^6/\mu\text{L}$ 3.96

HEMOGLOBIN g/dL 11.8*

HEMATOCRIT % 34.9

MCV fL 87.9

MCH pg 29.7

MCHC g/dL 33.7

RDW % 14.7

PLATELETS AUTO $10^3/\mu\text{L}$ 223

MPV fL 8.9

NEUTROPHILS RELATIVE % 47.1

LYMPHOCYTES RELATIVE % 40.9

MONOCYTES RELATIVE % 9.4*

EOSINOPHILS RELATIVE % 2.1

BASOPHILS RELATIVE % 0.5

NEUTROS ABS $10^3/\mu\text{L}$ 2.3

LYMPHS ABS AUTO $10^3/\mu\text{L}$ 2.0

MONOS ABS AUTO $10^3/\mu\text{L}$ 0.5

EOS ABS AUTO $10^3/\mu\text{L}$ 0.1

BASOS ABS AUTO $10^3/\mu\text{L}$ 0.0

CMP

Results from last 7 days

Lab Units 12/04/25

0421

SODIUM mmol/L 141

POTASSIUM mmol/L 3.6

CHLORIDE mmol/L 106

CO₂ mmol/L 26.0

BUN mg/dL 8.0

CREATININE mg/dL 0.80

BUN / CREAT RATIO 10.0

GLOMERULAR FILTRATION RATE mL/min/1.73m² 79.4*

MAGNESIUM mg/dL 1.90

CALCIUM UA mg/dL 9.2

Results from last 7 days

Lab Units 12/04/25

0421

GLUCOSE mg/dL 93

Cardiac

Results from last 7 days

Lab Units 12/04/25
0724 12/04/25
0421 12/03/25
2021
HS TROPONIN I ng/L 1,300* 1,366* 1,574*
BNP pg/mL -- 75.47 --

Coagulation

Thyroid Function / HGBA1C
Results from last 7 days
Lab Units 12/04/25
0421
TSH uIU/mL 1.342
HEMOGLOBIN A1C % 5.4

Urine

COVID-19

Results in Past 30 Days
Result Component Current Result Ref Range Previous Result Ref Range
SARS-CoV-2 by NAA Negative (12/4/2025) Negative Not in Time Range

Micro

No results found for: "AERCULTLC", "AFBCULTURE", "ANACX", "BLOODCX", "BONECX", "BRONCHIALCX", "CAMPY1", "CATHETERCX", "CAURISCX", "CRESCREENCX1", "CSFCX", "CULTURE", "CXBPRTUSSIS", "EARCULTURE", "EYECULTURE", "FUNG CX", "GCCX", "GENITALCULTU", "LABENVI", "LABFUNG", "LABGCC", "MRSACULTURE", "MYCOPLASCX" No results found for: "NASALCX", "NOSECULTURE", "RECT1", "SPUTUMCULTUR", "STOOLCX", "STREPACULT", "STREPACX", "STREPB CULT", "THROATCX", "TISSUECULTUR", "TRANREXCX1", "TRANSRXCX1", "UPPERGICX", "URINECX", "VRE CX", "WOUNDCX", "YERSINIACULT"

Imaging (last 48 hours)
X-ray Chest 1 View

Result Date: 12/4/2025
: 1. No acute chest disease. This document has been electronically signed by: Andrew Ciccarelli, MD on 12/04/2025 02:44:51

Assessment and Plan

NSTEMI

History of Takotsubo's cardiomyopathy

Dyslipidemia

- EKG showed sinus rhythm with no acute ST segment changes
- Troponin of 1500 > 1366 > 1300,
- Cardiology consulted, started on heparin drip, recommending echo followed by possible cardiac catheterization tomorrow
- Echo shows normal EF of 60-65% with no wall motion abnormalities or significant valvular pathology.
- LDL of 114, started patient on atorvastatin
- NPO at midnight for possible cardiac catheterization tomorrow

Code status: Full code

DVT prophylaxis: On heparin

Admit to Inpatient

Ordered at: 12/03/25 2343

Reason for IP Admission related to Chest Pain:

C. There are high-risk objective findings (State Finding(s)) and/or chest pain unrelieved/persists despite parenteral analgesics and optimal anti-anginal therapy

As it relates to Indication C, findings:

Abnormal Cardiac Bio Marker

Cardiac or Medical:

Cardiac Telemetry

Reason for Inpatient Admission:

Chest Pain

Inpatient Stay Expected Due to:

CM (Current Medical Needs)

Estimated length of stay (Inpatient stay is appropriate when the patient is expected to require a stay in the hospital that crosses at least 2 midnights.):

2 days

Level of Care:

Telemetry

Continued inpatient stay expected due to: HA (High Acuity)

Author: Manvi R Nagam, MD

Electronically signed by Manvi R Nagam, MD at 12/04/2025 8:42 PM PST

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Discharge Summaries - documented in this encounter

Manvi R Nagam, MD - 12/05/2025 5:53 PM PST

Formatting of this note is different from the original.

DISCHARGE SUMMARY

Name: Michelle Caruso MRN: 116253192

DOB: 2/19/1955 Acct: 730000366346

Age: 70 y.o. Admit Date: 12/3/2025 7:58 PM

Sex: female Attending: No att. providers found

PCP: Sarah Maples Location: SMRMC 4 NE TELEMETRY

Discharge Date: 12/5/2025 5:53 PM

Admitting Physician: Deepthi Ciddi, MD

Discharge Physician: Manvi R Nagam, MD

Discharge Diagnoses:

NSTEMI-likely secondary to coronary vasospasm

History of takotsubo cardiomyopathy

Dyslipidemia

Consult Orders

Cardiology

Micro

No results found for: "AERCULTLC", "AFBCULTURE", "ANACX", "BLOODCX", "BONECX", "BRONCHIALCX", "CAMPY1", "CATHETERCX", "CAURISCX", "CRESCREENCX1", "CSFCX", "CULTURE", "CXBPERTUSSIS", "EARCULTURE", "EYECULTURE", "FUNG CX", "GCCX", "GENITALCULTU", "LABENVI", "LABFUNG", "LABGCC", "MRSACULTURE", "MYCOPLASCX" No results found for: "NASALCX", "NOSECULTURE", "RECT1", "SPUTUMCULTUR", "STOOLCX", "STREPACULT", "STREPACX", "STREPB CULT", "THROATCX", "TISSUECULTUR", "TRANREXCX1", "TRANSRXCX1", "UPPERGICX", "URINECX", "VRE CX", "WOUNDCX", "YERSINIACULT"

Imaging

Cardiac Catheterization

Result Date: 12/5/2025

Procedure performed: Coronary and left ventricular cineangiography with left heart catheterization Indication:

Elevated troponin Procedure: Patient was transported to the cardiac cath lab in a fasting state. The right radial site was prepped and draped usual sterile fashion 1% lidocaine was used for local anesthesia. Under ultrasound guidance the right radial artery was cannulated using modified Seldinger technique. A six French sheath was inserted. Through this a six French Jacky catheter was used to perform right and left coronary angiography. Jacky catheter was removed over a guidewire and a pigtail catheter was placed through the left ventricle. Single plane left ventricular cineangiogram was performed and the pigtail catheter was removed. Sheath was removed and hemo band placed. There were no complications. Findings: Hemodynamics: Aortic pressure was 130/70 Left ventricular pressure was 130/15 There was no aortic valvular or subvalvular gradient demonstrated on pullback Coronary angiography: Left main coronary artery is short but widely patent. Proximal left anterior descending coronary artery is widely patent and free of disease. Mid left anterior descending coronary artery has mild calcific plaquing within it which narrows the vessel by 30% or less. Distal left anterior descending coronary artery is a large caliber widely patent vessel that wraps well around the left ventricular apex. First diagonal branch of the LAD is a small caliber vessel which is free of disease. Second diagonal branch is a slightly larger vessel which bifurcates it also was free of significant disease. Right coronary artery is a good-sized dominant vessel which is free of any significant disease. The PDA bifurcates early. Both limbs of the PDA are free of significant disease. Posterolateral branches a small terminal branch which is free of disease. Left ventricular cineangiogram demonstrates normal global left ventricular systolic function with ejection fraction estimated at 65%. No wall motion abnormalities are seen. Impression: 1. Mild calcific plaque of the mid LAD with no high-grade stenoses noted. 2. Otherwise, angiographically normal epicardial coronary arteries 3. Normal left ventricular systolic function with the ejection fraction estimated at 65% and normal left ventricular end-diastolic pressure

Echocardiogram Complete

Result Date: 12/4/2025

Pt. Name: CARUSO, MICHELLE Date of Exam: 12/04/2025 HR: 65 bpm Pt. Height: 66 in Rhythm: NSR Pt. Weight: 176 lb BP : 126/77 mmHg BSA : 1.89 m² Sonographer: Courtney McCrary, RDCS Referring Physician: GAUVAIN^ALLISON
Pt. Location: 463 ----- APPROVED REPORT ----- EXAM: Comprehensive 2D, Doppler, and color-flow
Echocardiogram Patient Location: 463 HR: 65 bpm Rhythm: NSR Indications 70 YOF presenting with dyspnea and shortness of breath. History of dyslipidemia, and takotsubo (April 2023). Left Ventricle The left ventricle is normal in size and systolic function with an ejection fraction of 60-65% by Simpson's biplane. There are no wall motion abnormalities noted. There is mild concentric left ventricular hypertrophy. There is grade 1 (impaired relaxation) diastolic dysfunction with normal LV filling pressures. There is no ventricular septal defect visualized. No left ventricle thrombus noted on this study. Right Ventricle The right ventricle is normal in size and function; TAPSE and S' of 1.8 cm and 16 cm/s. RVSP is indeterminate. Atria The left atrium is normal in size, measuring 15 mL/m². The interatrial septum is intact with no evidence for an atrial septal defect. The right atrium is normal in size, measuring 11 cm². Aortic Valve The aortic valve is normal in structure and function. No aortic regurgitation is present. There is no aortic valvular stenosis. Mitral Valve The mitral valve is normal in structure and function. There is trace mitral regurgitation. There is no mitral valve stenosis. Tricuspid Valve The tricuspid valve is normal in structure and function. There is no tricuspid valve stenosis. There is trace tricuspid regurgitation. Pulmonic Valve The pulmonic valve is normal in structure and function. There is no pulmonic valvular stenosis. There is trace pulmonic regurgitation. Great Vessels The aortic root is normal in size. The ascending aorta is normal in size. The IVC is normal in size and collapses >50% with inspiration. The pulmonary artery is normal. Pericardium There is no pericardial effusion. There is no pleural effusion. M-Mode Aorta/Left Atrium Ao Diameter 3.09 (2.2-3.7cm) Left Atrium 3.12 (2.5-4.0cm) LA/Ao Ratio 1.01 2D Right Ventricle/Outflow RV major dia 7.10 cm RV basal dia 3.00 cm RV mid dia 1.50 cm 2D Aorta/Left Atrium Sinuses of Valsalva 2.90 cm Sinotubular Junction 2.30 cm LAESV A4C (MOD) 18.49 ml Ascending Aorta 3.00 cm LAESV A2C (MOD) 39.23 ml LA Volume Index 15 ml/m² 2D Right Atrium/Inflow RA Major axis 4.20 cm RA minor axis 3.60 cm IVC 1.58 cm RA area 11.00 cm Left Ventricle SV (MOD) A4C 49.89 ml LVLd A4C 7.64 cm SV (MOD) A2C 33.38 ml LVLd A2C 7.00 cm LVEDV (MOD) BP 68.29 ml LVLs A4C 6.15 cm LVESV (MOD) BP 26.12 ml LVLs A2C 5.41 cm LVEF (MOD) A4C 64.80 % LVEDV A4C 77.00 ml LVEF (MOD) A2C 60.05 % LVEDV A2C 55.59 ml LVEF (MOD) BP 61.76 % LVESV A4C 27.11 ml LVESV A2C 22.21 ml Pulmonary Veins P Vein S Vel 66.48 cm/s P Vein a Vel 37.35 cm/s P Vein D Vel 40.43 cm/s P Vein a Dur 113.03 ms P Vein S/D Ratio 1.64 Mitral Inflow/TDI MV Peak E 59.00 cm/s DT 289.31 ms MV Peak A 85.62 cm/s E/e' Ratio 7.38 MV A Duration 108.42 ms IVRT 71.51 ms E/A Ratio 0.7 S' 16.00 cm/s e' Average 8.00 cm/s Right Ventricle TAPSE 1.80 cm Aortic Valve LVOT Dia 2.05 (1.8-2.4cm) AVA VTI 2.78 cm² LVOT Peak Vel 124.00 cm/s LVOT VTI 26.27 cm² AV Peak Vel 167.45 cm/s AV VTI 31.00 cm AV Mean Vel 104.00 cm/s AV Peak Grad 11.22 mmHg AV Mean Grad 5.25 mmHg Tricuspid Valve RAP 3.00 mmHg Pulmonary Valve PV Peak Vel 109.41 cm/s PV Peak Grad 4.79 mmHg PV Accel Time 147.64 ms Conclusion 1. The left ventricle is normal in size and systolic function with an ejection fraction of 60-65% by Simpson's biplane. There are no wall motion abnormalities noted. There is mild concentric left ventricular hypertrophy. There is grade 1 (impaired relaxation) diastolic dysfunction with normal LV filling pressures. 2. There are no significant valvular abnormalities. 3. The right ventricle is normal in size and function; RVSP is indeterminate. **No prior echocardiogram for comparison.

X-ray Chest 1 View

Result Date: 12/4/2025

REASON FOR STUDY: f/u pna? One view AP portable upright Chest Xray COMPARISON: None provided FINDINGS: Heart and mediastinum are within normal limits. Lungs are clear. No acute soft tissue or bony findings. Chronic left clavicle shaft fracture deformity.

: 1. No acute chest disease. This document has been electronically signed by: Andrew Ciccarelli, MD on 12/04/2025 02:44:51

HPI: " The patient is a 70-year-old female with a past medical history of takotsubo syndrome and dyslipidemia, who presented to Carson valley hospital complaining of waxing and waning dizziness and dyspnea since this morning. It is not associated with activity. She denies headache, diaphoresis, clamminess, palpitations, chest pain, cough, or nausea. She had stomach cramps for approximately half an hour yesterday and subsequently had a small amount of diarrhea, but none of the symptoms recurred. She is completely asymptomatic at present. She does endorse significant stress at work recently, had ever this does not feel anything like her previous episode of takotsubo. In Gardnerville, high sensitivity troponin was greater than 2000. Lab work was otherwise unremarkable. Chest x-ray

demonstrated possible right middle lung pneumonia. The patient was transferred to Saint Mary's for higher level of care. In our ED, EKG is sinus rhythm with T-wave inversions. High sensitivity troponin is 1574. "

Hospital Course: Patient was admitted to the hospital for NSTEMI in the setting of prior history of takotsubo cardiomyopathy. EKG upon admission showed no acute ST segment changes, however troponin was elevated at 1500 and trended down to 1366 > 1300. Cardiology was consulted and she was started on a heparin drip. Echocardiogram showed an EF of 60-65% with no wall motion abnormalities or significant valvular pathology. She underwent cardiac catheterization in the following day which showed no significant obstructive coronary artery disease. Her LDL was elevated at 111 and was started on atorvastatin. She was started on Cardizem for possible coronary vasospasm and continued on aspirin and Lipitor and discharged home in a stable condition. Cardiology is recommending outpatient cardiac MRI and patient understands that she needs to follow up with Cardiology as outpatient

Discharge instructions:

- The coronary angiogram showed no evidence of obstructive coronary artery disease and your echocardiogram showed no Takotsubo's cardiomyopathy.
- Cardiology is recommending outpatient cardiac MRI for complete workup
- Continue aspirin, Lipitor and Cardizem as prescribed
- Follow up with PCP and Cardiology as outpatient

Physical Exam

Constitutional:

General: She is not in acute distress.

Appearance: Normal appearance.

HENT:

Head: Normocephalic.

Eyes:

Extraocular Movements: Extraocular movements intact.

Conjunctiva/sclera: Conjunctivae normal.

Cardiovascular:

Rate and Rhythm: Normal rate and regular rhythm.

Heart sounds: Normal heart sounds.

Pulmonary:

Effort: No respiratory distress.

Breath sounds: Normal breath sounds. No wheezing.

Abdominal:

General: There is no distension.

Palpations: Abdomen is soft.

Tenderness: There is no abdominal tenderness.

Musculoskeletal:

Cervical back: Normal range of motion.

Right lower leg: No edema.

Left lower leg: No edema.

Skin:

Findings: No lesion or rash.

Neurological:

General: No focal deficit present.

Mental Status: She is alert and oriented to person, place, and time.

Psychiatric:

Mood and Affect: Mood normal.

Behavior: Behavior normal.

Discharge Medications

Medication List

START taking these medications

atorvastatin 40 MG tablet

Commonly known as: LIPITOR

Take 1 tablet (40 mg total) by mouth nightly for 30 days

dilTIAZem HCl ER 120 MG Tb24

Take 30 mg by mouth daily for 30 days

CONTINUE taking these medications

aspirin 81 MG EC tablet

Take 1 tablet (81 mg total) by mouth daily for 30 days

Where to Get Your Medications

These medications were sent to SMITH'S PHARMACY 70600392 - DAYTON, NV - 2200 HWY 50 E 2200 HWY 50 E, DAYTON NV 89403

Phone: 775-246-0920

aspirin 81 MG EC tablet

atorvastatin 40 MG tablet

dilTIAZem HCl ER 120 MG Tb24

Discharge Disposition:

Home or Self Care

Allergies

Allergies

Allergen Reactions

Citrus

Codeine GI Intolerance

Coffee Flavor [Flavoring Agent]

Gluten Meal

Vinegar [Acetic Acid]

Code Status: Full Code

Activity: As tolerated

Diet: Regular

Discharge Follow-Up: With PCP in two weeks

Time spent: Greater than 30 minutes

Admit to Inpatient

Ordered at: 12/03/25 2343

Level of Care:

Telemetry

Cardiac or Medical:

Cardiac Telemetry

Reason for Inpatient Admission:

Chest Pain

Reason for IP Admission related to Chest Pain:

C. There are high-risk objective findings (State Finding(s)) and/or chest pain unrelieved/persists despite parenteral analgesics and optimal anti-anginal therapy

As it relates to Indication C, findings:

Abnormal Cardiac Bio Marker

Inpatient Stay Expected Due to:

CM (Current Medical Needs)

Estimated length of stay (Inpatient stay is appropriate when the patient is expected to require a stay in the hospital that crosses at least 2 midnights.):

2 days

Author: Manvi R Nagam, MD

Electronically signed by Manvi R Nagam, MD at 12/08/2025 9:32 AM PST

Lab Results - documented in this encounter

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(ABNORMAL) Lipid panel (12/04/2025 4:21 AM PST)

Hemoglobin A1c (12/04/2025 4:21 AM PST)

(ABNORMAL) CBC w/Differential (12/04/2025 4:21 AM PST)

(ABNORMAL) Basic Metabolic Panel (12/04/2025 4:21 AM PST)

SARS-CoV-2/Influenza/RSV Molecular Assay (12/04/2025 2:49 AM PST)

Anti-Xa (Heparin Level) (12/03/2025 8:21 PM PST)

(ABNORMAL) TROPONIN I (12/03/2025 8:21 PM PST)

(ABNORMAL) TROPONIN I (12/05/2025 9:18 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HS Troponin I	1,102 (HH)	<34 ng/L		12/05/2025 9:48 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
HS Troponin I % Change	-15	%		12/05/2025 9:48 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
HS Troponin I ABS Change	-198			12/05/2025 9:48 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood		Venipuncture / Unknown	12/05/2025 9:18 AM PST	12/05/2025 9:24 AM PST

Narrative

ST MARY'S REGIONAL MED CTR LABORATORY - 12/05/2025 9:48 AM PST

High-sensitivity Troponin I (hsTnI) assay can reliably detect low troponin concentrations relative to conventional troponin assays. It is the preferred marker of myocardial necrosis as recommended by the Fourth Universal Definition of

Myocardial Infarction Guidelines.

The diagnosis of acute myocardial infarction (AMI) is made based on a rise or fall of troponin with at least one measurement exceeding the laboratory's upper limit of normal (indicating myocardial injury), in the context of reasonable suspicion for coronary ischemia (e.g. typical symptoms, changes on ECG, evidence for loss of myocardial function or demonstration of obstructive coronary artery disease).

NOTE: Although abnormal hsTnI values reflect INJURY to myocardial cells, an elevated hsTnI does not indicate the CAUSE of injury (i.e. ischemia versus non-ischemic disease).

In some cases, myocardial injury is chronic and relatively stable such that hsTnI values remain elevated but do not change substantially over hours to days (examples include chronic kidney disease, heart failure or advanced patient age).

When determining whether there has been a significant rise or fall of troponin on serial sampling, absolute change in troponin concentration has greater diagnostic accuracy for AMI than relative change criteria. A change of >7 ng/L over a 2-hour interval or a change of >10 ng/L over a 3-hour interval is suggested as a significant change. If the initial hsTnI is below or slightly above the 99th percentile upper reference limit, a 50% change from the baseline is considered significant. If the initial hsTnI is above the 99th percentile upper reference limit, a 20% change from the baseline value is considered significant.

Authorizing Provider	Result Type	Result Status	
Tracy L Birkner PA	LAB BLOOD ORDERABLES	Final Result	
Performing Organization	Address	City/State/ZIP Code	Phone Number
ST MARY'S REGIONAL MED CTR LABORATORY	235 West Sixth St	Reno, NV 89503-4548	775-770-3940

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COMPREHENSIVE METABOLIC PANEL (12/05/2025 4:37 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Pathologist Performed At	Signature
Sodium	140	136 - 145 mmol/L		12/05/2025 5:13 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
Potassium	4.1	3.5 - 5.3 mmol/L		12/05/2025 5:13 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
Chloride	108	98 - 110 mmol/L		12/05/2025 5:13 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
CO2	25.0	20.0 - 31.0 mmol/L		12/05/2025 5:13 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
BUN	7.0	6.0 - 24.0 mg/dL		12/05/2025 5:13 AM PST	ST MARY'S REGIONAL	

				MED CTR LABORATORY
Creatinine	0.70	0.50 - 1.00 mg/dL	12/05/2025 5:13 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
BUN/Creatinine Ratio	10.0	10.0 - 20.0	12/05/2025 5:13 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Glucose	97	70 - 140 mg/dL	12/05/2025 5:13 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Calcium	9.8	8.6 - 10.4 mg/dL	12/05/2025 5:13 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
AST	22	10 - 36 U/L	12/05/2025 5:13 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
ALT	10	6 - 29 U/L	12/05/2025 5:13 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Alkaline Phosphatase	64	33 - 130 U/L	12/05/2025 5:13 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Total Protein	6.6	6.4 - 8.4 g/dL	12/05/2025 5:13 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Albumin	4.1	3.6 - 5.1 g/dL	12/05/2025 5:13 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Total Bilirubin	0.5	0.2 - 1.2 mg/dL	12/05/2025 5:13 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
A/G Ratio	1.6	0.8 - 2.0	12/05/2025 5:13 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Anion Gap	7.0	6 - 19 mmol/L	12/05/2025 5:13 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Glomerular Filtration Rate	>90.0	>90.0 mL/min/1.73m*2	12/05/2025 5:13 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood		Venipuncture / Unknown	12/05/2025 4:37 AM PST	12/05/2025 4:49 AM PST

Narrative

ST MARY'S REGIONAL MED CTR LABORATORY - 12/05/2025 5:13 AM PST
STAGES OF CHRONIC KIDNEY DISEASE

STAGE	GFR	DESCRIPTION
1	90.0+	Normal kidney function but urine findings or structural abnormalities or genetic trait point to kidney disease.
2	60.0-89.0	Mildly reduced kidney function, and other findings (as in stage 1) point to kidney disease.
3	30.0-59.0	Moderately reduced kidney function.
4	15.0-29.0	Severely reduced kidney function.
5	<15.0	Very severe, or endstage kidney failure.

Authorizing Provider	Result Type	Result Status
Manvi R Nagam MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ST MARY'S REGIONAL MED CTR LABORATORY	235 West Sixth St	Reno, NV 89503- 4548	775-770-3940

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MAGNESIUM (12/05/2025 4:37 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Magnesium	1.90	1.50 - 2.50 mg/dL		12/05/2025 5:11 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood		Venipuncture / Unknown	12/05/2025 4:37 AM PST	12/05/2025 4:49 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Manvi R Nagam MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number

ST MARY'S REGIONAL MED CTR LABORATORY	235 West Sixth St	Reno, NV 89503-4548	775-770-3940
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PHOSPHORUS (12/05/2025 4:37 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Phosphorus	3.8	2.1 - 4.3 mg/dL		12/05/2025 5:13 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood		Venipuncture / Unknown	12/05/2025 4:37 AM PST	12/05/2025 4:49 AM PST

Narrative

ST MARY'S REGIONAL MED CTR LABORATORY - 12/05/2025 5:13 AM PST

High doses of Ambisome and Doxil therapy (or any other drug with delivery facilitated by a liposomal envelope) may cause falsely elevated results for Phosphorus.

Authorizing Provider	Result Type	Result Status
Manvi R Nagam MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ST MARY'S REGIONAL MED CTR LABORATORY	235 West Sixth St	Reno, NV 89503-4548	775-770-3940

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Anti-Xa (Heparin Level) (12/05/2025 4:37 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Heparin Anti-Xa	0.67	0.3 - 0.7 IU/mL		12/05/2025 5:16 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood		Venipuncture / Unknown	12/05/2025 4:37 AM PST	12/05/2025 4:49 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
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Allison E Shantz NP	LAB BLOOD ORDERABLES	Final Result	
Performing Organization	Address	City/State/ZIP Code	Phone Number
ST MARY'S REGIONAL MED CTR LABORATORY	235 West Sixth St	Reno, NV 89503-4548	775-770-3940

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(ABNORMAL) CBC W/DIFFERENTIAL (12/05/2025 4:36 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.60	3.40 - 10.00 10 ³ /uL		12/05/2025 5:11 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
RBC	4.04	3.90 - 5.03 10 ⁶ /uL		12/05/2025 5:11 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
Hemoglobin	12.2	12.0 - 15.5 g/dL		12/05/2025 5:11 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
Hematocrit	35.5	34.9 - 44.5 %		12/05/2025 5:11 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
MCV	87.9	81.6 - 98.3 fL		12/05/2025 5:11 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
MCH	30.2	27.5 - 33.2 pg		12/05/2025 5:11 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
MCHC	34.4	33.4 - 35.5 g/dL		12/05/2025 5:11 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
RDW	14.4	11.8 - 15.6 %		12/05/2025 5:11 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
Platelets	207	150 - 450 10 ³ /uL		12/05/2025 5:11 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
MPV	8.6	7.4 - 11.0 fL		12/05/2025 5:11 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	

Neutrophils (Relative)	45.0	42.0 - 75.0 %	12/05/2025 5:11 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Lymphocytes (Relative)	42.8	22.0 - 44.0 %	12/05/2025 5:11 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Monocytes (Relative)	9.6 (H)	2.0 - 9.0 %	12/05/2025 5:11 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Eosinophils (Relative)	2.1	1.0 - 7.0 %	12/05/2025 5:11 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Basophils (Relative)	0.5	0.0 - 1.0 %	12/05/2025 5:11 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Neutrophils (Absolute)	2.1	1.7 - 7.0 10 ³ /uL	12/05/2025 5:11 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Lymphocytes (Absolute)	2.0	0.9 - 2.9 10 ³ /uL	12/05/2025 5:11 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Monocytes (Absolute)	0.4	0.3 - 0.9 10 ³ /uL	12/05/2025 5:11 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Eosinophils (Absolute)	0.1	0.0 - 0.5 10 ³ /uL	12/05/2025 5:11 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Basophils (Absolute)	0.0	0.0 - 0.2 10 ³ /uL	12/05/2025 5:11 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood		Venipuncture / Unknown	12/05/2025 4:36 AM PST	12/05/2025 4:49 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Manvi R Nagam MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
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ST MARY'S REGIONAL MED CTR LABORATORY	235 West Sixth St	Reno, NV 89503-4548	775-770-3940
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Anti-Xa (Heparin Level) (12/04/2025 6:21 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Heparin Anti-Xa	0.63	0.3 - 0.7 IU/mL		12/04/2025 6:51 PM PST	ST MARY'S REGIONAL MED CTR LABORATORY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood		Venipuncture / Unknown	12/04/2025 6:21 PM PST	12/04/2025 6:38 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Manvi R Nagam MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ST MARY'S REGIONAL MED CTR LABORATORY	235 West Sixth St	Reno, NV 89503-4548	775-770-3940

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Anti-Xa (Heparin Level) (12/04/2025 3:06 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Heparin Anti-Xa	0.64	0.3 - 0.7 IU/mL		12/04/2025 3:52 PM PST	ST MARY'S REGIONAL MED CTR LABORATORY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood		Venipuncture / Unknown	12/04/2025 3:06 PM PST	12/04/2025 3:21 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Manvi R Nagam MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ST MARY'S REGIONAL MED CTR LABORATORY	235 West Sixth St	Reno, NV 89503-4548	775-770-3940

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(ABNORMAL) Anti-Xa (Heparin Level) (12/04/2025 7:24 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Heparin Anti-Xa	0.74 (H)	0.3 - 0.7 IU/mL		12/04/2025 8:31 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood		Venipuncture / Unknown	12/04/2025 7:24 AM PST	12/04/2025 8:16 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
John E Stewart MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ST MARY'S REGIONAL MED CTR LABORATORY	235 West Sixth St	Reno, NV 89503-4548	775-770-3940

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(ABNORMAL) Troponin I (12/04/2025 7:24 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HS Troponin I	1,300 (HH)	<34 ng/L		12/04/2025 8:45 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
HS Troponin I % Change	-5	%		12/04/2025 8:45 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
HS Troponin I ABS Change	-66			12/04/2025 8:45 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood		Venipuncture / Unknown	12/04/2025 7:24 AM PST	12/04/2025 8:16 AM PST

Narrative

ST MARY'S REGIONAL MED CTR LABORATORY - 12/04/2025 8:45 AM PST

High-sensitivity Troponin I (hsTnI) assay can reliably detect low troponin concentrations relative to conventional troponin assays. It is the preferred marker of myocardial necrosis as recommended by the Fourth Universal Definition of Myocardial Infarction Guidelines.

The diagnosis of acute myocardial infarction (AMI) is made based on a rise or fall of troponin with at least one measurement exceeding the laboratory's upper limit of normal (indicating myocardial injury), in the context of reasonable suspicion for coronary ischemia (e.g. typical symptoms, changes on ECG, evidence for loss of myocardial function or demonstration of obstructive coronary artery disease).

NOTE: Although abnormal hsTnI values reflect INJURY to myocardial cells, an elevated hsTnI does not indicate the CAUSE of injury (i.e. ischemia versus non-ischemic disease).

In some cases, myocardial injury is chronic and relatively stable such that hsTnI values remain elevated but do not change substantially over hours to days (examples include chronic kidney disease, heart failure or advanced patient age).

When determining whether there has been a significant rise or fall of troponin on serial sampling, absolute change in troponin concentration has greater diagnostic accuracy for AMI than relative change criteria. A change of >7 ng/L over a 2-hour interval or a change of >10 ng/L over a 3-hour interval is suggested as a significant change. If the initial hsTnI is below or slightly above the 99th percentile upper reference limit, a 50% change from the baseline is considered significant. If the initial hsTnI is above the 99th percentile upper reference limit, a 20% change from the baseline value is considered significant.

Authorizing Provider	Result Type	Result Status
Allison E Shantz NP	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ST MARY'S REGIONAL MED CTR LABORATORY	235 West Sixth St	Reno, NV 89503- 4548	775-770-3940

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Anti-Xa (Heparin Level) (12/04/2025 4:21 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Heparin Anti-Xa	0.58	0.3 - 0.7 IU/mL		12/04/2025 5:00 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood		Venipuncture / Unknown	12/04/2025 4:21 AM PST	12/04/2025 4:45 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
John E Stewart MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ST MARY'S REGIONAL MED CTR LABORATORY	235 West Sixth St	Reno, NV 89503-4548	775-770-3940

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TSH Rfx on Abnormal to Free T4 (12/04/2025 4:21 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
TSH	1.342	0.400 - 4.500 uIU/mL		12/04/2025 5:07 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood		Venipuncture / Unknown	12/04/2025 4:21 AM PST	12/04/2025 4:45 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Allison E Shantz NP	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ST MARY'S REGIONAL MED CTR LABORATORY	235 West Sixth St	Reno, NV 89503-4548	775-770-3940

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Procalcitonin Test (12/04/2025 4:21 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Procalcitonin	<0.04	0.00 - 0.15 ng/mL		12/04/2025 5:13 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	

Comment: Systemic infection (sepsis) is not likely.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood		Venipuncture / Unknown	12/04/2025 4:21 AM PST	12/04/2025 4:45 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Allison E Shantz NP	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ST MARY'S REGIONAL MED CTR LABORATORY	235 West Sixth St	Reno, NV 89503- 4548	775-770-3940

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Magnesium (12/04/2025 4:21 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Magnesium	1.90	1.50 - 2.50 mg/dL		12/04/2025 5:05 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood		Venipuncture / Unknown	12/04/2025 4:21 AM PST	12/04/2025 4:45 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Allison E Shantz NP	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ST MARY'S REGIONAL MED CTR LABORATORY	235 West Sixth St	Reno, NV 89503- 4548	775-770-3940

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B-TYPE NATRIURETIC PEPTIDE (12/04/2025 4:21 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
BNP	75.47	<=100 pg/mL		12/04/2025 5:04 AM	ST MARY'S REGIONAL	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood		Venipuncture / Unknown	12/04/2025 4:21 AM PST	12/04/2025 4:45 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Allison E Shantz NP	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ST MARY'S REGIONAL MED CTR LABORATORY	235 West Sixth St	Reno, NV 89503- 4548	775-770-3940

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(ABNORMAL) Troponin I (12/04/2025 4:21 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HS Troponin I	1,366 (HH)	<34 ng/L		12/04/2025 5:21 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
HS Troponin I % Change	-13	%		12/04/2025 5:21 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
HS Troponin I ABS Change	-208			12/04/2025 5:21 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood		Venipuncture / Unknown	12/04/2025 4:21 AM PST	12/04/2025 4:45 AM PST

Narrative

ST MARY'S REGIONAL MED CTR LABORATORY - 12/04/2025 5:21 AM PST
High-sensitivity Troponin I (hsTnI) assay can reliably detect low troponin concentrations relative to conventional troponin assays. It is the preferred marker of myocardial necrosis as recommended by the Fourth Universal Definition of Myocardial Infarction Guidelines.

The diagnosis of acute myocardial infarction (AMI) is made based on a rise or fall of troponin with at least one measurement exceeding the laboratory's upper limit of normal(indicating myocardial injury), in the context of reasonable suspicion for coronary ischemia (e.g. typical symptoms, changes on ECG, evidence for loss of myocardial function or demonstration of obstructive coronary artery disease).

NOTE: Although abnormal hsTnI values reflect INJURY to myocardial cells, an elevated hsTnI does not indicate the CAUSE of injury (i.e. ischemia versus non-ischemic disease).

In some cases, myocardial injury is chronic and relatively stable such that hsTnI values remain elevated but do not change substantially over hours to days (examples include chronic kidney disease, heart failure or advanced patient age).

When determining whether there has been a significant rise or fall of troponin on serial sampling, absolute change in troponin concentration has greater diagnostic accuracy for AMI than relative change criteria. A change of >7 ng/L over a 2-hour interval or a change of >10 ng/L over a 3-hour interval is suggested as a significant change. If the initial hsTnI is below or slightly above the 99th percentile upper reference limit, a 50% change from the baseline is considered significant. If the initial hsTnI is above the 99th percentile upper reference limit, a 20% change from the baseline value is considered significant.

Authorizing Provider	Result Type	Result Status
Allison E Shantz NP	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ST MARY'S REGIONAL MED CTR LABORATORY	235 West Sixth St	Reno, NV 89503-4548	775-770-3940

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(ABNORMAL) Lipid panel (12/04/2025 4:21 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Cholesterol	198	125 - 200 mg/dL		12/04/2025 5:07 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
Triglycerides	48	0 - 150 mg/dL		12/04/2025 5:07 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
HDL	74.0	>=50.0 mg/dL		12/04/2025 5:07 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
LDL CHOLESTEROL CALCULATED	114 (H)	<=100 mg/dL		12/04/2025 5:07 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
Cardiac Risk	2.7	0.1 - 5.0 RATIO		12/04/2025 5:07 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
VLDL	9.6	6 - 30 mg/dL		12/04/2025 5:07 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood		Venipuncture / Unknown	12/04/2025 4:21 AM PST	12/04/2025 4:45 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Allison E Shantz NP	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ST MARY'S REGIONAL MED CTR LABORATORY	235 West Sixth St	Reno, NV 89503- 4548	775-770-3940

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Hemoglobin A1c (12/04/2025 4:21 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Hgb A1c	5.4	4.0 - 5.6 %		12/04/2025 4:58 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood		Venipuncture / Unknown	12/04/2025 4:21 AM PST	12/04/2025 4:45 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Allison E Shantz NP	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ST MARY'S REGIONAL MED CTR LABORATORY	235 West Sixth St	Reno, NV 89503- 4548	775-770-3940

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(ABNORMAL) CBC w/Differential (12/04/2025 4:21 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	4.90	3.40 - 10.00		12/04/2025 4:58 AM	ST MARY'S REGIONAL	

		10*3/uL	PST	MED CTR LABORATORY
RBC	3.96	3.90 - 5.03 10*6/uL	12/04/2025 4:58 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Hemoglobin	11.8 (L)	12.0 - 15.5 g/dL	12/04/2025 4:58 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Hematocrit	34.9	34.9 - 44.5 %	12/04/2025 4:58 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
MCV	87.9	81.6 - 98.3 fL	12/04/2025 4:58 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
MCH	29.7	27.5 - 33.2 pg	12/04/2025 4:58 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
MCHC	33.7	33.4 - 35.5 g/dL	12/04/2025 4:58 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
RDW	14.7	11.8 - 15.6 %	12/04/2025 4:58 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Platelets	223	150 - 450 10*3/uL	12/04/2025 4:58 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
MPV	8.9	7.4 - 11.0 fL	12/04/2025 4:58 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Neutrophils (Relative)	47.1	42.0 - 75.0 %	12/04/2025 4:58 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Lymphocytes (Relative)	40.9	22.0 - 44.0 %	12/04/2025 4:58 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Monocytes (Relative)	9.4 (H)	2.0 - 9.0 %	12/04/2025 4:58 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Eosinophils (Relative)	2.1	1.0 - 7.0 %	12/04/2025 4:58 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY

Basophils (Relative)	0.5	0.0 - 1.0 %	12/04/2025 4:58 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Neutrophils (Absolute)	2.3	1.7 - 7.0 10*3/uL	12/04/2025 4:58 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Lymphocytes (Absolute)	2.0	0.9 - 2.9 10*3/uL	12/04/2025 4:58 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Monocytes (Absolute)	0.5	0.3 - 0.9 10*3/uL	12/04/2025 4:58 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Eosinophils (Absolute)	0.1	0.0 - 0.5 10*3/uL	12/04/2025 4:58 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Basophils (Absolute)	0.0	0.0 - 0.2 10*3/uL	12/04/2025 4:58 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood		Venipuncture / Unknown	12/04/2025 4:21 AM PST	12/04/2025 4:45 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Allison E Shantz NP	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ST MARY'S REGIONAL MED CTR LABORATORY	235 West Sixth St	Reno, NV 89503-4548	775-770-3940

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(ABNORMAL) Basic Metabolic Panel (12/04/2025 4:21 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sodium	141	136 - 145 mmol/L		12/04/2025 5:05 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
Potassium	3.6	3.5 - 5.3 mmol/L		12/04/2025 5:05 AM	ST MARY'S REGIONAL	

			PST	MED CTR LABORATORY
Chloride	106	98 - 110 mmol/L	12/04/2025 5:05 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
CO2	26.0	20.0 - 31.0 mmol/L	12/04/2025 5:05 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
BUN	8.0	6.0 - 24.0 mg/dL	12/04/2025 5:05 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Creatinine	0.80	0.50 - 1.00 mg/dL	12/04/2025 5:05 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Glucose	93	70 - 140 mg/dL	12/04/2025 5:05 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Calcium	9.2	8.6 - 10.4 mg/dL	12/04/2025 5:05 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Glomerular Filtration Rate	79.4 (L)	>90.0 mL/min/1.73m*2	12/04/2025 5:05 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
Anion Gap	9.0	6 - 19 mmol/L	12/04/2025 5:05 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY
BUN/Creatinine Ratio	10.0	10.0 - 20.0	12/04/2025 5:05 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood		Venipuncture / Unknown	12/04/2025 4:21 AM PST	12/04/2025 4:45 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Allison E Shantz NP	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ST MARY'S REGIONAL MED CTR	235 West Sixth St	Reno, NV 89503- 4548	775-770-3940

LABORATORY

[Back to top of Lab Results](#)**SARS-CoV-2/Influenza/RSV Molecular Assay (12/04/2025 2:49 AM PST)**

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
SARS-CoV-2 by NAA	Negative	Negative		12/04/2025 6:29 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
Influenza A	Negative	Negative		12/04/2025 6:29 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
Influenza B	Negative	Negative		12/04/2025 6:29 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	
Respiratory Syncytial Virus	Negative	Negative		12/04/2025 6:29 AM PST	ST MARY'S REGIONAL MED CTR LABORATORY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Nasopharyngeal	Nasopharyngeal swab / Unknown	Collection / Unknown	12/04/2025 2:49 AM PST	12/04/2025 3:38 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Allison E Shantz NP	MICROBIOLOGY - GENERAL ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ST MARY'S REGIONAL MED CTR LABORATORY	235 West Sixth St	Reno, NV 89503-4548	775-770-3940

[Back to top of Lab Results](#)**Anti-Xa (Heparin Level) (12/03/2025 8:21 PM PST)**

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Heparin Anti-Xa	0.56	0.3 - 0.7 IU/mL		12/03/2025 8:43 PM PST	ST MARY'S REGIONAL MED CTR LABORATORY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood		Venipuncture / Unknown	12/03/2025 8:21 PM PST	12/03/2025 8:28 PM PST

Narrative

Authorizing Provider	Result Type	Result Status
Richard M Law MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ST MARY'S REGIONAL MED CTR LABORATORY	235 West Sixth St	Reno, NV 89503- 4548	775-770-3940

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(ABNORMAL) TROPONIN I (12/03/2025 8:21 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
HS Troponin I	1,574 (HH)	<34 ng/L		12/03/2025 9:00 PM PST	ST MARY'S REGIONAL MED CTR LABORATORY	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood		Venipuncture / Unknown	12/03/2025 8:21 PM PST	12/03/2025 8:28 PM PST

Narrative

ST MARY'S REGIONAL MED CTR LABORATORY - 12/03/2025 9:00 PM PST
High-sensitivity Troponin I (hsTnI) assay can reliably detect low troponin concentrations relative to conventional troponin assays. It is the preferred marker of myocardial necrosis as recommended by the Fourth Universal Definition of Myocardial Infarction Guidelines.

The diagnosis of acute myocardial infarction (AMI) is made based on a rise or fall of troponin with at least one measurement exceeding the laboratory's upper limit of normal (indicating myocardial injury), in the context of reasonable suspicion for coronary ischemia (e.g. typical symptoms, changes on ECG, evidence for loss of myocardial function or demonstration of obstructive coronary artery disease).

NOTE: Although abnormal hsTnI values reflect INJURY to myocardial cells, an elevated hsTnI does not indicate the CAUSE of injury (i.e. ischemia versus non-ischemic disease).

In some cases, myocardial injury is chronic and relatively stable such that hsTnI values remain elevated but do not change substantially over hours to days (examples include chronic kidney disease, heart failure or advanced patient age).

When determining whether there has been a significant rise or fall of troponin on serial sampling, absolute change in troponin concentration has greater diagnostic accuracy for AMI than relative change criteria. A change of >7 ng/L over a 2-hour interval or a change of >10 ng/L over a 3-hour interval is suggested as a significant change. If the initial hsTnI is below or slightly above the 99th percentile upper reference limit, a 50% change from the baseline is considered significant. If the

initial hsTnI is above the 99th percentile upper reference limit, a 20% change from the baseline value is considered significant.

Authorizing Provider	Result Type	Result Status
Richard M Law MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ST MARY'S REGIONAL MED CTR LABORATORY	235 West Sixth St	Reno, NV 89503-4548	775-770-3940

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Imaging Results - documented in this encounter

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[LEFT HEART CATH - RADIAL \(12/05/2025 2:47 PM PST\)](#)

[ECHOCARDIOGRAM COMPLETE \(93306\) \(12/04/2025 12:04 PM PST\)](#)

[X-ray Chest 1 View \(12/04/2025 12:24 AM PST\)](#)

LEFT HEART CATH - RADIAL (12/05/2025 2:47 PM PST)

Anatomical Region	Laterality	Modality		
Cardiac Catheterization				
Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time

Narrative

12/05/2025 3:05 PM PST

Procedure performed: Coronary and left ventricular cineangiography with left heart catheterization

Indication: Elevated troponin

Procedure:

Patient was transported to the cardiac cath lab in a fasting state. The right radial site was prepped and draped usual sterile fashion 1% lidocaine was used for local anesthesia. Under ultrasound guidance the right radial artery was cannulated using modified Seldinger technique. A six French sheath was inserted. Through this a six French Jacky catheter was used to perform right and left coronary angiography. Jacky catheter was removed over a guidewire and a pigtail catheter was placed through the left ventricle. Single plane left ventricular cineangiogram was performed and the pigtail catheter was removed. Sheath was removed and hemo band placed. There were no complications.

Findings:

Hemodynamics:

Aortic pressure was 130/70

Left ventricular pressure was 130/15

There was no aortic valvular or subvalvular gradient demonstrated on pullback

Coronary angiography:

Left main coronary artery is short but widely patent.

Proximal left anterior descending coronary artery is widely patent and free of disease.

Mid left anterior descending coronary artery has mild calcific plaquing within it which narrows the vessel by 30% or less.

Distal left anterior descending coronary artery is a large caliber widely patent vessel that wraps well around the left ventricular apex.

First diagonal branch of the LAD is a small caliber vessel which is free of disease. Second diagonal branch is a slightly larger vessel which bifurcates it also was free of significant disease.

Right coronary artery is a good-sized dominant vessel which is free of any significant disease. The PDA bifurcates early. Both limbs of the PDA are free of significant disease. Posterolateral branches a small terminal branch which is free of disease.

Left ventricular cineangiogram demonstrates normal global left ventricular systolic function with ejection fraction estimated at 65%. No wall motion abnormalities are seen.

Impression:

1. Mild calcific plaque of the mid LAD with no high-grade stenoses noted.
2. Otherwise, angiographically normal epicardial coronary arteries
3. Normal left ventricular systolic function with the ejection fraction estimated at 65% and normal left ventricular end-diastolic pressure

Procedure Details

Procedure performed: Coronary and left ventricular cineangiography with left heart catheterization

Indication: Elevated troponin

Procedure:

Patient was transported to the cardiac cath lab in a fasting state. The right radial site was prepped and draped usual sterile fashion 1% lidocaine was used for local anesthesia. Under ultrasound guidance the right radial artery was cannulated using modified Seldinger technique. A six French sheath was inserted. Through this a six French Jacky catheter was used to perform right and left coronary angiography. Jacky catheter was removed over a guidewire and a pigtail catheter was placed through the left ventricle. Single plane left ventricular cineangiogram was performed and the pigtail catheter was removed. Sheath was removed and hemo band placed. There were no complications.

Findings:

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First diagonal branch of the LAD is a small caliber vessel which is free of disease.

Second diagonal branch is a slightly larger vessel which bifurcates it also was free of significant disease.

Right coronary artery is a good-sized dominant vessel which is free of any significant disease. The PDA bifurcates early. Both limbs of the PDA are free of significant disease. Posterolateral branches a small terminal branch which is free of disease.

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2. Otherwise, angiographically normal epicardial coronary arteries
3. Normal left ventricular systolic function with the ejection fraction estimated at 65% and normal left ventricular end-diastolic pressure

Authorizing Provider	Result Type	Result Status
Tracy L Birkner PA	CV CARDIAC CATH ORDERABLES	Final Result

ECHOCARDIOGRAM COMPLETE (93306) (12/04/2025 12:04 PM PST)

Anatomical Region	Laterality	Modality
		Echocardiogram

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			12/04/2025 1:46 PM PST	

Narrative

12/04/2025 1:46 PM PST

Pt. Name: CARUSO, MICHELLE Date of Exam: 12/04/2025

HR: 65 bpm Pt. Height: 66 in

Rhythm: NSR Pt. Weight: 176 lb

BP : 126/77 mmHg BSA : 1.89 m2

Sonographer: Courtney McCrary, RDCS

Referring Physician: GAUVAIN^ALLISON

Pt. Location: 463

----- APPROVED REPORT -----

EXAM: Comprehensive 2D, Doppler, and color-flow Echocardiogram

Patient Location: 463

HR: 65 bpm

Rhythm: NSR

Indications

70 YOF presenting with dyspnea and shortness of breath. History of dyslipidemia, and takotsubo (April 2023).

Left Ventricle

The left ventricle is normal in size and systolic function with an ejection fraction of 60-65% by Simpson's biplane. There are no wall motion abnormalities noted. There is mild concentric left ventricular hypertrophy. There is grade 1 (impaired relaxation) diastolic dysfunction with normal LV filling pressures. There is no ventricular septal defect visualized. No left ventricle thrombus noted on this study.

Right Ventricle

The right ventricle is normal in size and function; TAPSE and S' of 1.8 cm and 16 cm/s. RVSP is indeterminate.

Atria

The left atrium is normal in size, measuring 15 mL/m2. The interatrial septum is intact with no evidence for an atrial septal defect. The right atrium is normal in size, measuring 11 cm2.

Aortic Valve

The aortic valve is normal in structure and function. No aortic regurgitation is present. There is no aortic valvular stenosis.

Mitral Valve

The mitral valve is normal in structure and function. There is trace mitral regurgitation. There is no mitral valve stenosis.

Tricuspid Valve

The tricuspid valve is normal in structure and function. There is no tricuspid valve stenosis. There is trace tricuspid regurgitation.

Pulmonic Valve

The pulmonic valve is normal in structure and function. There is no pulmonic valvular

stenosis. There is trace pulmonic regurgitation.

Great vessels

The aortic root is normal in size. The ascending aorta is normal in size. The IVC is normal in size and collapses >50% with inspiration. The pulmonary artery is normal.

Pericardium

There is no pericardial effusion. There is no pleural effusion.

M-Mode Aorta/Left Atrium

Ao Diameter 3.09 (2.2-3.7cm) Left Atrium 3.12 (2.5-4.0cm) LA/Ao Ratio 1.01

2D Right Ventricle/Outflow

RV major dia 7.10 cm RV basal dia 3.00 cm

RV mid dia 1.50 cm

2D Aorta/Left Atrium

Sinuses of Valsalva 2.90 cm Sinotubular Junction 2.30 cm

LAESV A4C (MOD) 18.49 ml Ascending Aorta 3.00 cm

LAESV A2C (MOD) 39.23 ml LA Volume Index 15 ml/m²

2D Right Atrium/Inflow

RA Major axis 4.20 cm RA minor axis 3.60 cm

IVC 1.58 cm RA area 11.00 cm

Left Ventricle

SV (MOD) A4C 49.89 ml LVLd A4C 7.64 cm

SV (MOD) A2C 33.38 ml LVLd A2C 7.00 cm

LVEDV (MOD) BP 68.29 ml LVLs A4C 6.15 cm

LVESV (MOD) BP 26.12 ml LVLs A2C 5.41 cm

LVEF (MOD) A4C 64.80 % LVEDV A4C 77.00 ml

LVEF (MOD) A2C 60.05 % LVEDV A2C 55.59 ml

LVEF (MOD) BP 61.76 % LVESV A4C 27.11 ml

LVESV A2C 22.21 ml

Pulmonary Veins

P Vein S Vel 66.48 cm/s P Vein a Vel 37.35 cm/s

P Vein D Vel 40.43 cm/s P Vein a Dur 113.03 ms

P Vein S/D Ratio 1.64

Mitral Inflow/TDI

MV Peak E 59.00 cm/s DT 289.31 ms

MV Peak A 85.62 cm/s E/e' Ratio 7.38

MV A Duration 108.42 ms IVRT 71.51 ms

E/A Ratio 0.7 S' 16.00 cm/s

e' Average 8.00 cm/s

Right Ventricle

TAPSE 1.80 cm

Aortic Valve

LVOT Dia 2.05 (1.8-2.4cm) AVA VTI 2.78 cm²

LVOT Peak Vel 124.00 cm/s LVOT VTI 26.27 cm²

AV Peak Vel 167.45 cm/s AV VTI 31.00 cm

AV Mean Vel 104.00 cm/s AV Peak Grad 11.22 mmHg

AV Mean Grad 5.25 mmHg

Tricuspid Valve

RAP 3.00 mmHg

Pulmonary Valve

PV Peak Vel 109.41 cm/s PV Peak Grad 4.79 mmHg

PV Accel Time 147.64 ms

Conclusion

1. The left ventricle is normal in size and systolic function with an ejection fraction of 60-65% by Simpson's biplane. There are no wall motion abnormalities noted. There is mild concentric left ventricular hypertrophy. There is grade 1 (impaired relaxation) diastolic dysfunction with normal LV filling pressures.
2. There are no significant valvular abnormalities.

3. The right ventricle is normal in size and function; RVSP is indeterminate.

****No prior echocardiogram for comparison.**

Procedure Note

Abu Sheikha, Mohammad M, MD - 12/04/2025

Formatting of this note might be different from the original.

Pt. Name: CARUSO, MICHELLE Date of Exam: 12/04/2025

HR: 65 bpm Pt. Height: 66 in

Rhythm: NSR Pt. Weight: 176 lb

BP : 126/77 mmHg BSA : 1.89 m²

Sonographer: Courtney McCrary, RDCS

Referring Physician: GAUVAIN^ALLISON

Pt. Location: 463

----- APPROVED REPORT -----

EXAM: Comprehensive 2D, Doppler, and color-flow Echocardiogram

Patient Location: 463

HR: 65 bpm

Rhythm: NSR

Indications

70 YOF presenting with dyspnea and shortness of breath. History of dyslipidemia, and takotsubo (April 2023).

Left Ventricle

The left ventricle is normal in size and systolic function with an ejection fraction of 60-65% by Simpson's biplane. There are no wall motion abnormalities noted. There is mild concentric left ventricular hypertrophy. There is grade 1 (impaired relaxation) diastolic dysfunction with normal LV filling pressures. There is no ventricular septal defect visualized. No left ventricle thrombus noted on this study.

Right Ventricle

The right ventricle is normal in size and function; TAPSE and S' of 1.8 cm and 16 cm/s. RVSP is indeterminate.

Atria

The left atrium is normal in size, measuring 15 mL/m². The interatrial septum is intact with no evidence for an atrial septal defect. The right atrium is normal in size, measuring 11 cm².

Aortic Valve

The aortic valve is normal in structure and function. No aortic regurgitation is present. There is no aortic valvular stenosis.

Mitral Valve

The mitral valve is normal in structure and function. There is trace mitral regurgitation. There is no mitral valve stenosis.

Tricuspid Valve

The tricuspid valve is normal in structure and function. There is no tricuspid valve stenosis. There is trace tricuspid regurgitation.

Pulmonic Valve

The pulmonic valve is normal in structure and function. There is no pulmonic valvular stenosis. There is trace pulmonic regurgitation.

Great Vessels

The aortic root is normal in size. The ascending aorta is normal in size. The IVC is normal in size and collapses >50% with inspiration. The pulmonary artery is normal.

Pericardium

There is no pericardial effusion. There is no pleural effusion.

M-Mode Aorta/Left Atrium

Ao Diameter 3.09 (2.2-3.7cm) Left Atrium 3.12 (2.5-4.0cm) LA/Ao Ratio 1.01

2D Right Ventricle/Outflow

RV major dia 7.10 cm RV basal dia 3.00 cm

RV mid dia 1.50 cm

2D Aorta/Left Atrium

Sinuses of Valsalva 2.90 cm Sinotubular Junction 2.30 cm

LAESV A4C (MOD) 18.49 ml Ascending Aorta 3.00 cm

LAESV A2C (MOD) 39.23 ml LA Volume Index 15 ml/m²

2D Right Atrium/Inflow

RA Major axis 4.20 cm RA minor axis 3.60 cm

IVC 1.58 cm RA area 11.00 cm

Left Ventricle

SV (MOD) A4C 49.89 ml LVLd A4C 7.64 cm

SV (MOD) A2C 33.38 ml LVLd A2C 7.00 cm

LVEDV (MOD) BP 68.29 ml LVLs A4C 6.15 cm

LVESV (MOD) BP 26.12 ml LVLs A2C 5.41 cm

LVEF (MOD) A4C 64.80 % LVEDV A4C 77.00 ml

LVEF (MOD) A2C 60.05 % LVEDV A2C 55.59 ml

LVEF (MOD) BP 61.76 % LVESV A4C 27.11 ml

LVESV A2C 22.21 ml

Pulmonary Veins

P Vein S Vel 66.48 cm/s P Vein a Vel 37.35 cm/s

P Vein D Vel 40.43 cm/s P Vein a Dur 113.03 ms

P Vein S/D Ratio 1.64

Mitral Inflow/TDI

MV Peak E 59.00 cm/s DT 289.31 ms

MV Peak A 85.62 cm/s E/e' Ratio 7.38

MV A Duration 108.42 ms IVRT 71.51 ms

E/A Ratio 0.7 S' 16.00 cm/s

e' Average 8.00 cm/s

Right Ventricle

TAPSE 1.80 cm

Aortic Valve

LVOT Dia 2.05 (1.8-2.4cm) AVA VTI 2.78 cm²

LVOT Peak Vel 124.00 cm/s LVOT VTI 26.27 cm²

AV Peak Vel 167.45 cm/s AV VTI 31.00 cm

AV Mean Vel 104.00 cm/s AV Peak Grad 11.22 mmHg

AV Mean Grad 5.25 mmHg

Tricuspid Valve

RAP 3.00 mmHg

Pulmonary Valve

PV Peak Vel 109.41 cm/s PV Peak Grad 4.79 mmHg

PV Accel Time 147.64 ms

Conclusion

1. The left ventricle is normal in size and systolic function with an ejection fraction of 60-65% by Simpson's biplane. There are no wall motion abnormalities noted. There is mild concentric left ventricular hypertrophy. There is grade 1 (impaired relaxation) diastolic dysfunction with normal LV filling pressures.
2. There are no significant valvular abnormalities.
3. The right ventricle is normal in size and function; RVSP is indeterminate.

****No prior echocardiogram for comparison.**

Authorizing Provider	Result Type	Result Status
Allison E Shantz NP	CV ECHO ORDERABLES	Final Result

[Back to top of Imaging Results](#)

X-ray Chest 1 View (12/04/2025 12:24 AM PST)

Anatomical Region	Laterality	Modality
Chest	N/A	Radiographic Imaging

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			12/04/2025 2:44 AM PST	

Impressions

12/04/2025 12:45 AM PST

:

1. No acute chest disease.

This document has been electronically signed by: Andrew Ciccarelli, MD on
12/04/2025 02:44:51

Narrative

12/04/2025 12:45 AM PST

REASON FOR STUDY: f/u pna?

One view AP portable upright Chest Xray

COMPARISON: None provided

FINDINGS:

Heart and mediastinum are within normal limits. Lungs are clear. No acute soft tissue or bony findings. Chronic left clavicle shaft fracture deformity.

Procedure Note

Ciccarelli, Andrew M, MD - 12/04/2025

Formatting of this note might be different from the original.
REASON FOR STUDY: f/u pna?

One view AP portable upright Chest Xray

COMPARISON: None provided

FINDINGS:

Heart and mediastinum are within normal limits. Lungs are clear. No acute soft tissue or bony findings. Chronic left clavicle shaft fracture deformity.

IMPRESSION

- :
- 1. No acute chest disease.

This document has been electronically signed by: Andrew Ciccarelli, MD on 12/04/2025 02:44:51

Authorizing Provider	Result Type	Result Status
Allison E Shantz NP	IMG DIAGNOSTIC IMAGING ORDERABLES	Final Result

[Back to top of Imaging Results](#)

Procedures - documented in this encounter

Procedure Name	Priority	Date/Time	Associated Diagnosis	Comments
CARDIAC CATHETERIZATION	Routine	12/05/2025 2:47 PM PST		Results for this procedure are in the results section .
TROPONIN I	Add-On	12/05/2025 9:18 AM PST		Results for this procedure are in the results section .
ANTI-XA	Routine	12/05/2025 4:37 AM PST		Results for this procedure are in the results section .
PHOSPHORUS	Routine	12/05/2025 4:37 AM PST		Results for this procedure are in the results section .
MAGNESIUM, SERUM	Routine	12/05/2025 4:37 AM PST		Results for this procedure are in the results section .
COMPREHENSIVE METABOLIC PANEL	Routine	12/05/2025 4:37 AM PST		Results for this procedure are in the results section .
CBC W/ AUTO DIFF	Routine	12/05/2025 4:36 AM PST		Results for this procedure are in the results section .
ANTI-XA	Routine	12/04/2025 6:21 PM PST		Results for this procedure are in the results section .
ANTI-XA	Routine	12/04/2025 3:06 PM PST		Results for this procedure are in the results section .
ECHOCARDIOGRAM COMPLETE (93306)	Routine	12/04/2025 12:04 PM PST		Results for this procedure are in the results section .
ANTI-XA	Routine	12/04/2025 7:24 AM PST		Results for this procedure are in the results section .
TROPONIN I	Timed	12/04/2025 7:24 AM PST		Results for this procedure are in the results section .
TSH RFX ON ABNORMAL TO FREE T4	Add-On	12/04/2025 4:21 AM PST		Results for this procedure are in the results section .
ANTI-XA	Timed	12/04/2025 4:21 AM PST		Results for this procedure are in the results section .
PROCALCITONIN TEST	Add-On	12/04/2025 4:21 AM PST		Results for this procedure are in the results section .
TROPONIN I	Timed	12/04/2025 4:21 AM PST		Results for this procedure are in the results section .
CBC W/ AUTO DIFF	Routine	12/04/2025 4:21 AM PST		Results for this procedure are in the results section .
B-TYPE NATRIURETIC PEPTIDE	Routine	12/04/2025 4:21 AM PST		Results for this procedure are in the results section .
MAGNESIUM, SERUM	Routine	12/04/2025 4:21 AM PST		Results for this procedure are in the results section .
HEMOGLOBIN A1C	Routine	12/04/2025 4:21 AM PST		Results for this procedure are in the results section .
LIPID PANEL	Routine	12/04/2025 4:21 AM PST		Results for this procedure are in the results section .
BASIC METABOLIC PANEL	Routine	12/04/2025 4:21 AM PST		Results for this procedure are in the results section .
SARS-COV-2/INFLUENZA/RSV MOLECULAR ASSAY	STAT	12/04/2025 2:49 AM PST		Results for this procedure are in the results section .
XR CHEST 1 VIEW	STAT	12/04/2025 12:24 AM PST		Results for this procedure are in the results section .

ANTI-XA	Timed	12/03/2025 8:21 PM PST	Results for this procedure are in the results section .
TROPONIN I	STAT	12/03/2025 8:21 PM PST	Results for this procedure are in the results section .
ECG 12-LEAD	Routine	12/03/2025 8:06 PM PST	Results for this procedure are in the results section .

Reason for Referral

(Routine) - Incomplete

Specialty		Diagnoses / Procedures	Referred By Contact		Referred To Contact	
		Procedures Case Request to Cath Lab: Left Heart Cath - Radial	Birkner, Tracy L, PA Phone: tel:+1-775-302-0000 fax:+1-775-260-0368			
Referral ID	Status	Reason	Start Date	Expiration Date	Visits Requested	Visits Authorized
12434774	Incomplete		12/5/2025	12/5/2026	1	1

Electronically signed by Birkner, Tracy L, PA at 12/05/2025 7:55 AM PST

(Routine) - Incomplete

Specialty		Diagnoses / Procedures	Referred By Contact		Referred To Contact	
		Procedures Echocardiogram Complete	Shantz, Allison E, NP Phone: tel:+1-775-770-6490 fax:+1-775-770-3944			
Referral ID	Status	Reason	Start Date	Expiration Date	Visits Requested	Visits Authorized
12419357	Incomplete		12/3/2025	12/3/2026	1	1

Electronically signed by Shantz, Allison E, NP at 12/03/2025 11:43 PM PST

Reason for Visit

Reason	Comments
Dizziness	Pt BIBA with CC of SOB and dizziness which began earlier today and has since has resolved. Per EMS Pt had a troponin at 2000 PTA.
Shortness of Breath	

Auth/Cert (Routine)			
Specialty	Diagnoses / Procedures	Referred By Contact	Referred To Contact
	Diagnoses NSTEMI (non-ST elevated myocardial infarction) (CMS/HCC)		
	Procedures PR HOSPITAL IP/OBS CARE SAME DATE SF/LOW MDM 45 MIN		

Referral ID	Status	Reason	Start Date	Expiration Date	Visits Requested	Visits Authorized
12424877					1	1

Encounter Details

Date	Type	Department	Care Team (Latest Contact Info)	Description
12/03/2025 7:58 PM PST - 12/05/2025 5:53 PM PST	Hospital Encounter	SMRMC 4 NE TELEMETRY 236 W 6TH STREET RENO, NV 89503 775-770-3388	Van Bibber, Daniel B, MD 235 West Sixth Street, ER Dept. Reno, NV 89503 775-770-3188 (Work) 775-770-3588 (Fax)	NSTEMI (non-ST elevated myocardial infarction) (CMS/HCC) (Primary Dx) Discharge Disposition: Home or Self Care
			Stewart, John E, MD 411 West Sixth Street Reno, NV 89503 775-770-6490 (Work) 775-770-3944 (Fax)	
			Nagam, Manvi R, MD 411 West Sixth Street Reno, NV 89503 775-770-6490 (Work) 775-770-3944 (Fax)	
			Allen, Ladrina, RN	
			Lachino, Saul, RN	
			Shantz, Allison E, NP 411 West Sixth Street Reno, NV 89503 775-770-6490 (Work) 775-770-3944 (Fax)	
			Reno, Antone, RN	
			Segura, Ingrid, CNA	
			Fairman, Lois, RN	
			Ventura, Hyacinth, RN	
			Behm, Marlana, Discharge Planner	
			Heiter, Sarah, RN	
			Carrea, Frank P, MD Frank P. Carrea, MD Independent Interventional Cardiology Coverage Reno, NV 89511-4780 775-338-9815 (Work)	
			Ciddi, Deepthi, MD 411 West Sixth Street Reno, NV 89503 775-770-6490 (Work)	

Social History - documented as of this encounter

Tobacco Use	Types	Packs/Day	Years Used	Date
Smoking Tobacco: Never				
Smokeless Tobacco: Never				
Alcohol Use	Standard Drinks/Week	Comments		
Yes	0 (1 standard drink = 0.6 oz pure alcohol)	5 per year		
B1300 Health Literacy		Answer	Date Recorded	
How often do you need to have someone help you when you read instructions, pamphlets, or other written material from your doctor or pharmacy?		Never	12/04/2025	
AHC Utilities		Answer	Date Recorded	
In the past 12 months has the electric, gas, oil, or water company threatened to shut off services in your home?		No	12/04/2025	
Humiliation, Afraid, Rape, and Kick questionnaire		Answer	Date Recorded	
Within the last year, have you been afraid of your partner or ex-partner?		No	12/04/2025	
Within the last year, have you been humiliated or emotionally abused in other ways by your partner or ex-partner?		No	12/04/2025	
Within the last year, have you been kicked, hit, slapped, or otherwise physically hurt by your partner or ex-partner?		No	12/04/2025	
Within the last year, have you been raped or forced to have any kind of sexual activity by your partner or ex-partner?		No	12/04/2025	
Social Connection and Isolation Panel		Answer	Date Recorded	
In a typical week, how many times do you talk on the phone with family, friends, or neighbors?		Three times a week	12/04/2025	
How often do you get together with friends or relatives?		Three times a week	12/04/2025	
How often do you attend church or religious services?		Never	12/04/2025	
Do you belong to any clubs or organizations such as church groups, unions, fraternal or athletic groups, or school groups?		No	12/04/2025	
How often do you attend meetings of the clubs or organizations you belong to?		Never	12/04/2025	
Are you married, widowed, divorced, separated, never married, or living with a partner?		Divorced	12/04/2025	
AUDIT-C		Answer	Date Recorded	
Q1: How often do you have a drink containing alcohol?		Monthly or less	12/04/2025	
Q2: How many drinks containing alcohol do you have on a typical day when you are drinking?		1 or 2	12/04/2025	
Q3: How often do you have six or more drinks on one occasion?		Never	12/04/2025	
Overall Financial Resource Strain (CARDIA)		Answer	Date Recorded	

How hard is it for you to pay for the very basics like food, housing, medical care, and heating?	Not hard at all	12/04/2025
PHQ-2	Answer	Date Recorded
PHQ 2 Total	1	12/04/2025
Finnish Institute of Occupational Health - Occupational Stress Questionnaire	Answer	Date Recorded
Do you feel stress - tense, restless, nervous, or anxious, or unable to sleep at night because your mind is troubled all the time - these days?	To some extent	12/04/2025
Exercise Vital Sign	Answer	Date Recorded
On average, how many days per week do you engage in moderate to strenuous exercise (like a brisk walk)?	2 days	12/04/2025
On average, how many minutes do you engage in exercise at this level?	10 min	12/04/2025
Hunger Vital Sign	Answer	Date Recorded
Within the past 12 months, you worried that your food would run out before you got the money to buy more.	Never true	12/04/2025
Within the past 12 months, the food you bought just didn't last and you didn't have money to get more.	Never true	12/04/2025
PRAPARE - Transportation	Answer	Date Recorded
In the past 12 months, has lack of transportation kept you from medical appointments or from getting medications?	No	12/04/2025
In the past 12 months, has lack of transportation kept you from meetings, work, or from getting things needed for daily living?	No	12/04/2025
Housing Stability Vital Sign	Answer	Date Recorded
In the last 12 months, was there a time when you were not able to pay the mortgage or rent on time?	Yes	12/04/2025
In the past 12 months, how many times have you moved where you were living?	1	12/04/2025
At any time in the past 12 months, were you homeless or living in a shelter (including now)?	No	12/04/2025
Pregnant	Comments	
Unknown		
Sex and Gender Information	Value	Date Recorded
Sex Assigned at Birth	Female	03/04/2025 8:31 AM PST
Legal Sex	Female	02/17/2025 3:29 PM PST
Gender Identity	Female	03/04/2025 8:31 AM PST
Sexual Orientation	Straight	03/04/2025 8:31 AM PST

Last Filed Vital Signs - documented in this encounter

Vital Sign	Reading	Time Taken	Comments
Blood Pressure	126/65	12/05/2025 5:00 PM PST	
Pulse	66	12/05/2025 5:00 PM PST	
Temperature	37.3 °C (99.1 °F)	12/05/2025 3:00 PM PST	
Respiratory Rate	16	12/05/2025 3:00 PM PST	
Oxygen Saturation	93%	12/05/2025 3:30 PM PST	
Inhaled Oxygen Concentration	-	-	
Weight	80 kg (176 lb 5.9 oz)	12/05/2025 12:26 AM PST	
Height	167.6 cm (5' 6")	12/04/2025 3:00 AM PST	
Body Mass Index	28.47	12/04/2025 3:00 AM PST	

Functional Status - documented as of this encounter**Morse Fall Risk**

Question	Answer	Date of Assessment	Author
History of Falling (Immediate or Within 3 Months)	25	12/05/2025 8:05 AM PST	Ventura, Hyacinth, RN
Secondary Diagnosis	0	12/05/2025 8:05 AM PST	Ventura, Hyacinth, RN
Ambulatory Aids	0	12/05/2025 8:05 AM PST	Ventura, Hyacinth, RN
Intravenous Therapy/Heparin/Saline Lock	20	12/05/2025 8:05 AM PST	Ventura, Hyacinth, RN
Gait/Transferring	0	12/05/2025 8:05 AM PST	Ventura, Hyacinth, RN
Mental Status	0	12/05/2025 8:05 AM PST	Ventura, Hyacinth, RN
Score	45	12/05/2025 8:05 AM PST	Ventura, Hyacinth, RN
Morse Fall Risk Interventions	Falls Risk Armband band applied/in place;Proper non-slip footwear in place;Falls Risk identifier in place on door frame;Bed in lowest position;Bed/Chair alarm in place;Universal fall precautions	12/05/2025 8:05 AM PST	Ventura, Hyacinth, RN
Morse Fall Risk Level	High (45 and higher)	12/05/2025 8:05 AM PST	Ventura, Hyacinth, RN

Braden Scale

Question	Answer	Date of Assessment	Author
Sensory Perceptions	4	12/05/2025 8:05 AM PST	Ventura, Hyacinth, RN
Moisture	4	12/05/2025 8:05 AM PST	Ventura, Hyacinth, RN
Activity	4	12/05/2025 8:05 AM PST	Ventura, Hyacinth, RN
Mobility	4	12/05/2025 8:05 AM PST	Ventura, Hyacinth, RN
Nutrition	2	12/05/2025 8:05 AM PST	Ventura, Hyacinth, RN
Friction and Shear	2	12/05/2025 8:05 AM PST	Ventura, Hyacinth, RN
Braden Scale Score	20	12/05/2025 8:05 AM PST	Ventura, Hyacinth, RN

Universal Fall Risk Interventions

Question	Answer	Date of Assessment	Author
Falls Risk Armband band applied/in place	Yes	12/05/2025 8:05 AM PST	Ventura, Hyacinth, RN
Fall Injury Prevention Protocol	Yes	12/05/2025 8:05 AM PST	Ventura, Hyacinth, RN

AUDIT-C Score

Answer	Date of Assessment	Author
1	12/04/2025 4:37 AM PST	Reno, Antone, RN

Skip to questions 9-10?

Answer	Date of Assessment	Author
1	12/04/2025 4:37 AM PST	Reno, Antone, RN

Alcohol Use

Question	Answer	Date of Assessment	Author
Q1: How often do you have a drink containing alcohol?	Monthly or less	12/04/2025 4:37 AM PST	Reno, Antone, RN
Q2: How many drinks containing alcohol do you have on a typical day when you are drinking?	1 or 2	12/04/2025 4:37 AM PST	Reno, Antone, RN
Q3: How often do you have six or more drinks on one occasion?	Never	12/04/2025 4:37 AM PST	Reno, Antone, RN

Fall Risk Calculated Score

Answer	Date of Assessment	Author
45 (Morse)	12/05/2025 8:05 AM PST	Ventura, Hyacinth, RN

Gravely Disabled

Question	Answer	Date of Assessment	Author
Severely Neglected Personal Hygiene- ADLs	No	12/03/2025 8:08 PM PST	Lachino, Saul, RN

ADL Screening

Question	Answer	Date of Assessment	Author
Patient's Vision Allow to Safely Complete Daily Activities	Yes	12/04/2025 3:00 AM PST	Reno, Antone, RN
Patient's Judgement Allow to Safely Complete Daily Activities	Yes	12/04/2025 3:00 AM PST	Reno, Antone, RN
Patient's Memory Allow to Safely Complete Daily Activities	Yes	12/04/2025 3:00 AM PST	Reno, Antone, RN
Patient Able to Express Needs/Desires	Yes	12/04/2025 3:00 AM PST	Reno, Antone, RN
Dressing	Independent	12/04/2025 3:00 AM PST	Reno, Antone, RN
Grooming	Independent	12/04/2025 3:00 AM PST	Reno, Antone, RN

Feeding	Independent	12/04/2025 3:00 AM PST	Reno, Antone, RN
Bathing	Independent	12/04/2025 3:00 AM PST	Reno, Antone, RN
Toileting	Independent	12/04/2025 3:00 AM PST	Reno, Antone, RN
In/Out Bed	Independent	12/04/2025 3:00 AM PST	Reno, Antone, RN
Walks in Home	Independent	12/04/2025 3:00 AM PST	Reno, Antone, RN
Weakness of Legs	None	12/04/2025 3:00 AM PST	Reno, Antone, RN
Weakness of Arms/Hands	None	12/04/2025 3:00 AM PST	Reno, Antone, RN
Hearing - Right Ear	Functional	12/04/2025 3:00 AM PST	Reno, Antone, RN
Hearing - Left Ear	Functional	12/04/2025 3:00 AM PST	Reno, Antone, RN
Which is your dominant hand?	Right	12/04/2025 3:00 AM PST	Reno, Antone, RN

Assistive Devices

Question	Answer	Date of Assessment	Author
Assistive Devices	Eyeglasses;Toilet riser	12/04/2025 3:00 AM PST	Reno, Antone, RN

Mental Status - documented in this encounter

Neurological

Question	Answer	Entry Date	Author
Orientation Level	Oriented X4	12/05/2025 8:05 AM PST	Ventura, Hyacinth, RN

Universal Fall Risk Interventions

Question	Answer	Entry Date	Author
Surroundings Orientation and Organization reviewed	Yes	12/05/2025 8:05 AM PST	Ventura, Hyacinth, RN

PHQ-9 Total Score

Answer	Entry Date	Author
1	12/04/2025 4:00 AM PST	Reno, Antone, RN

Over the last 2 weeks, how often have you been bothered by any of the following problems?

Question	Answer	Entry Date	Author
PHQ 2 Total	1	12/04/2025 4:00 AM PST	Reno, Antone, RN
Little interest or pleasure in doing things	0	12/04/2025 4:00 AM PST	Reno, Antone, RN
Feeling down, depressed, irritable, or hopeless	1	12/04/2025 4:00 AM PST	Reno, Antone, RN

Total Score

Answer	Entry Date	Author
1	12/04/2025 4:00 AM PST	Reno, Antone, RN

Is Patient Conscious/Coherent And Able To Participate?

Answer	Entry Date	Author
Yes	12/04/2025 4:00 AM PST	Reno, Antone, RN

Total Severity Score (Mood)

Answer	Entry Date	Author
1	12/04/2025 4:00 AM PST	Reno, Antone, RN

Discharge Instructions - documented in this encounter

Table of Contents for Discharge Instructions

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[Discharge Instr - Activity](#)

[Discharge Instr - Diet](#)

[Discharge Instr - Other Orders](#)

[Attachments](#)

Discharge Instructions

Regina Rayne Santos, RN - 12/05/2025 4:58 PM PST

Formatting of this note might be different from the original.

The ASCVD Risk score (Arnett DK, et al., 2019) failed to calculate for the following reasons:

The patient has a prior MI or stroke diagnosis

Electronically signed by Regina Rayne Santos, RN at 12/05/2025 4:58 PM PST

[Back to top of Discharge Instructions](#)

Discharge Instr - Activity

Regina Rayne Santos, RN - 12/05/2025 4:57 PM PST

Formatting of this note might be different from the original.

No lifting anything greater than 10lbs x 7 days. Showers okay. No soaking incision site

Electronically signed by Regina Rayne Santos, RN at 12/05/2025 4:57 PM PST

[Back to top of Discharge Instructions](#)

Discharge Instr - Diet

Regina Rayne Santos, RN - 12/05/2025 4:57 PM PST

Formatting of this note is different from the original.

Follow a heart healthy diet:

Low in saturated fats/cholesterol

Low in sodium

Wide variety of fruits/vegetables/grains

Electronically signed by Regina Rayne Santos, RN at 12/05/2025 4:57 PM PST

[Back to top of Discharge Instructions](#)

Discharge Instr - Other Orders

Regina Rayne Santos, RN - 12/05/2025 4:57 PM PST

Formatting of this note might be different from the original.

Please return to the ER or call 911 if you are experiencing:

Chest Pain

Shortness of breath

Slurred speech

Palpitations/Rapid heart rate

Increased swelling/Rapid weight gain

Electronically signed by Regina Rayne Santos, RN at 12/05/2025 4:57 PM PST

[Back to top of Discharge Instructions](#)

Attachments

The following attachments cannot be sent through Care Everywhere.

Heart Attack - Warning Signs and What to Do (English)

Coronary Artery Disease Female (English)

Coronary Angiogram (English)

Radial Site Care (English)

Cardiac Rehabilitation (English)

[Back to top of Discharge Instructions](#)

Medications at Time of Discharge - documented as of this encounter

Medication	Sig	Dispense Quantity	Refills	Last Filled	Start Date	End Date
aspirin 81 MG EC tablet	Take 1 tablet (81 mg total) by mouth daily for 30 days	30 tablet			12/05/2025	01/04/2026
atorvastatin (LIPITOR) 40 MG tablet	Take 1 tablet (40 mg total) by mouth nightly for 30 days	30 tablet			12/05/2025	01/04/2026
dilTIAZem HCl ER 120 MG TB24	Take 30 mg by mouth daily for 30 days	30 tablet			12/05/2025	12/15/2025

Ordered Prescriptions - documented in this encounter

Prescription	Sig	Dispense Quantity	Refills	Last Filled	Start Date	End Date
dilTIAZem HCl ER 120 MG TB24	Take 30 mg by mouth daily for 30 days	30 tablet			12/05/2025	12/15/2025
atorvastatin (LIPITOR) 40 MG tablet	Take 1 tablet (40 mg total) by mouth nightly for 30 days	30 tablet			12/05/2025	01/04/2026
aspirin 81 MG EC tablet	Take 1 tablet (81 mg total) by mouth daily for 30 days	30 tablet			12/05/2025	01/04/2026

Discharge Disposition - documented in this encounter

Disposition	Code	Departure Means	Destination	Comments
Home or Self Care		Car	Home	

H&P Notes - documented in this encounter

Allison E Shantz, NP - 12/03/2025 11:43 PM PST

Formatting of this note is different from the original.

Images from the original note were not included.

Michelle Caruso

70 y.o.

female

116253192

CHIEF COMPLAINT: Dizziness, shortness of breath

CONSULTATIONS: Dr. Carrea, cardiology

HPI: The patient is a 70-year-old female with a past medical history of takotsubo syndrome and dyslipidemia, who presented to Carson valley hospital complaining of waxing and waning dizziness and dyspnea since this morning. It is not associated with activity. She denies headache, diaphoresis, clamminess, palpitations, chest pain, cough, or nausea. She had stomach cramps for approximately half an hour yesterday and subsequently had a small amount of diarrhea, but none of the symptoms recurred. She is completely asymptomatic at present. She does endorse significant stress at work recently, had ever this does not feel anything like her previous episode of takotsubo. In Gardnerville, high sensitivity troponin was greater than 2000. Lab work was otherwise unremarkable. Chest x-ray demonstrated possible right middle lung pneumonia. The patient was transferred to Saint Mary's for higher level of care. In our ED, EKG is sinus rhythm with T-wave inversions. High sensitivity troponin is 1574.

Review of systems is otherwise negative by CMS and AMA criteria except as described above.

PAST MEDICAL HISTORY:

1. Takotsubo syndrome, April 2023
2. Dyslipidemia, not on medication
3. Osteoarthritis

PAST SURGICAL HISTORY:

1. Right THA, March 2025
2. Right shoulder surgery
3. Ectopic pregnancy
4. D&C
5. Eye surgery

FAMILY HISTORY:

Significant for heart disease in the patient's father

SOCIAL HISTORY:

The patient does not smoke cigarettes. She rarely drinks alcohol. She does not use drugs.

OUTPATIENT MEDICATIONS:

None

Results

Procedure Component Value Units Date/Time

Anti-Xa (Heparin Level) [470194968] (Normal) Collected: 12/03/25 2021

Specimen: Blood Updated: 12/03/25 2043

Heparin Anti-Xa 0.56 IU/mL

TROPONIN I [470194971] (Abnormal) Collected: 12/03/25 2021

Specimen: Blood Updated: 12/03/25 2100

HS Troponin I 1,574 ng/L

Narrative:

High-sensitivity Troponin I (hsTnI) assay can reliably detect low troponin concentrations relative to conventional

troponin assays. It is the preferred marker of myocardial necrosis as recommended by the Fourth Universal Definition of Myocardial Infarction Guidelines.

The diagnosis of acute myocardial infarction (AMI) is made based on a rise or fall of troponin with at least one measurement exceeding the laboratory's upper limit of normal (indicating myocardial injury), in the context of reasonable suspicion for coronary ischemia (e.g. typical symptoms, changes on ECG, evidence for loss of myocardial function or demonstration of obstructive coronary artery disease).

NOTE: Although abnormal hsTnI values reflect INJURY to myocardial cells, an elevated hsTnI does not indicate the CAUSE of injury (i.e. ischemia versus non-ischemic disease).

In some cases, myocardial injury is chronic and relatively stable such that hsTnI values remain elevated but do not change substantially over hours to days (examples include chronic kidney disease, heart failure or advanced patient age).

When determining whether there has been a significant rise or fall of troponin on serial sampling, absolute change in troponin concentration has greater diagnostic accuracy for AMI than relative change criteria. A change of >7 ng/L over a 2-hour interval or a change of >10 ng/L over a 3-hour interval is suggested as a significant change. If the initial hsTnI is below or slightly above the 99th percentile upper reference limit, a 50% change from the baseline is considered significant. If the initial hsTnI is above the 99th percentile upper reference limit, a 20% change from the baseline value is considered significant.

ESTIMATED GFR [470194937] Collected: 12/03/25 1549

Updated: 12/03/25 1959

TROPONIN [470194938] (Abnormal) Collected: 12/03/25 1549

Updated: 12/03/25 1959

COMP METABOLIC PANEL [470194939] Collected: 12/03/25 1549

Updated: 12/03/25 1959

CBC WITH DIFFERENTIAL [470194940] (Abnormal) Collected: 12/03/25 1549

Updated: 12/03/25 1959

APTT [470194941] (Abnormal) Collected: 12/03/25 1547

Updated: 12/03/25 1959

Radiology Results (last 2 days)

**** No results found for the last 48 hours. ****

Vitals:

12/03/25 2300

BP: 126/77

Pulse: 63

Resp: 13

Temp:

SpO2: 94%

PHYSICAL EXAM:

GENERAL: No acute distress

HEENT: Atraumatic and normocephalic

CARDIOVASCULAR: Regular rate and rhythm, no murmurs rubs or gallops. No peripheral edema.

PULMONARY: Lungs are clear to auscultation bilaterally, respirations regular and unlabored

GI: Abdomen is soft, nontender to palpation

MUSCULOSKELETAL: No chest wall tenderness to palpation. Joints and range of motion grossly normal

NEUROLOGIC: Patient is alert and oriented

ASSESSMENT:

1. Non STEMI
2. Possible right middle lung pneumonia
3. Dyslipidemia, not on medication
4. History of takotsubo syndrome

DISCUSSION AND PLAN:

The patient is being admitted to the cardiac telemetry unit. Cardiology has been consulted and will see her in the morning. She has been started on a heparin drip and we will continue to trend her troponins serially. A1c and lipid panel are pending. We will also check BNP and echocardiogram given her history. We will start her on antibiotics for possible pneumonia and repeat chest x-ray in the morning. Procalcitonin is pending. PRN medication will be available for pain, nausea, and hypertension.

VTE prophylaxis: Heparin drip

CODE STATUS: Full code

Anticipate 2 midnights of hospitalization due to possible ACS, continuous cardiac monitoring, serial lab work, NPO status, IV antibiotics

Admit to Inpatient

Ordered at: 12/03/25 2343

Reason for IP Admission related to Chest Pain:

C. There are high-risk objective findings (State Finding(s)) and/or chest pain unrelieved/persists despite parenteral analgesics and optimal anti-anginal therapy

As it relates to Indication C, findings:

Abnormal Cardiac Bio Marker

Cardiac or Medical:

Cardiac Telemetry

Reason for Inpatient Admission:

Chest Pain

Inpatient Stay Expected Due to:

CM (Current Medical Needs)

Estimated length of stay (Inpatient stay is appropriate when the patient is expected to require a stay in the hospital that crosses at least 2 midnights.):

2 days

Level of Care:

Telemetry

Electronically signed by Allison E Shantz, NP at 12/03/2025 11:51 PM PST

Consult Notes - documented in this encounter

Mohammad M Abu Sheikha, MD - 12/04/2025 9:44 AM PST

Formatting of this note is different from the original.
Images from the original note were not included.

CARDIOLOGY - INPATIENT CONSULT NOTE

Name: Michelle Caruso MRN: 116253192
DoB: 2/19/1955 Age: 70 y.o.

LENGTH OF STAY: 1

DATE OF CONSULT: 12/04/2025

Reason for Consult: NSTEMI

History of Present Illness:

70 y/o F still works, PMH of mild CAD (30% LAD stenosis), hx of NSTEMI/MINOCA due to Takutsubo 7/2024 (OCT showed no SCAD, it was probably Takutsubo vs vasospasm vs microvascular dysfunction), moderate MR, and HFpEF, whom cardiology is consulted for NSTEMI.

Patient did not present with CP, she has a lot of stress at work, she presented with dizziness and SOB.

She has not been taking ASA or statin or BB/CCB/Nitro or Lasix and instead taking supplements.

Labs showed HsT ~3000.

PAST MEDICAL HISTORY:

Past Medical History:
Diagnosis Date
Broken heart syndrome
Interstitial cystitis

PAST SURGICAL HISTORY:

Past Surgical History:
Procedure Laterality Date
CYSTOSCOPY
DILATION AND CURETTAGE OF UTERUS
LAPAROSCOPY FOR ECTOPIC PREGNANCY
PR ARTHRP ACETBLR/PROX FEM PROSTC AGRFT/ALGRFT Right 3/4/2025
Procedure: RIGHT TOTAL HIP ARTHROPLASTY, DIRECT ANTERIOR APPROACH; Surgeon: Jessell M Owens, MD;
Location: NV SMRMC OR; Service: Orthopedics Surgery
SHOULDER SURGERY Right

FAMILY HISTORY:

As per HPI.

SOCIAL HISTORY:

As per HPI.

CURRENT HOSPITAL MEDICATIONS:

Scheduled Meds:
aspirin 81 mg Oral Daily
cefTRIAxone (ROCEPHIN) IVPB 1 g Intravenous Q24H
doxycycline 100 mg Intravenous Q12H
Pharmacy to dose Heparin 1 Consult Does not apply See Admin Instructions

Continuous Infusions:
heparin 12 Units/kg/hr 11 Units/kg/hr (12/04/25 0922)

ALLERGIES:

Allergies
Allergen Reactions
Citrus
Codeine GI Intolerance
Coffee Flavor [Flavoring Agent]
Gluten Meal
Vinegar [Acetic Acid]

Review of Systems:

CONSTITUTIONAL: Negative for chills, fatigue, and fever.
EARS, NOSE, MOUTH, THROAT and FACE: Negative for epistaxis, snoring, and sore throat.
RESPIRATORY: Negative for cough, sneezing, or wheezing.
CARDIOVASCULAR: +Dizziness, +SOB
GASTROINTESTINAL: Negative for dysphagia, epigastric pain, and reflux symptoms.
GENITOURINARY: Negative for hematuria, increased frequency, and nocturia.
HEMATOLOGIC/LYMPHATIC: Negative for bleeding, easy bruising, and petechiae.
MUSCULOSKELETAL: Negative for arthralgias, muscle weakness, and myalgias.
NEUROLOGICAL: Negative for dizziness, headaches, and weakness.
BEHAVIORAL/PSYCH: Negative for anxiety, depression, and hallucinations.

Physical Exam:

Vitals:

Vitals:
12/04/25 0700
BP: (!) 144/80
Pulse: 59
Resp: 18
Temp: 97.7 °F (36.5 °C)
SpO2: 96%

CONSTITUTIONAL: Alert, oriented, well-appearing, well developed, well nourished, no distress.
RESPIRATORY: Clear to auscultation bilaterally.
CARDIOVASCULAR: Regular rate and rhythm. No murmurs. Abdominal aorta pulsation normal. Carotid, femoral, and pedal pulses 4+ bilaterally. No lower extremity edema.
GASTROINTESTINAL: Soft, non tender, without masses.
MUSCULOSKELETAL: Extremities have intact strength.
SKIN: Warm and dry, intact.
NEUROLOGICAL: Alert, oriented, normal speech.
PSYCHIATRIC: Alert and oriented, appropriate affect.

Data Review:

Intake and Output:

Intake/Output Summary (Last 24 hours) at 12/4/2025 0944
Last data filed at 12/4/2025 0600
Gross per 24 hour
Intake 0 ml
Output 0 ml
Net 0 ml

Labs:
Results

Procedure Component Value Units Date/Time
Troponin I [560794880] (Abnormal) Collected: 12/04/25 0724
Specimen: Blood Updated: 12/04/25 0845
HS Troponin I 1,300 ng/L
HS Troponin I % Change -5 %
HS Troponin I ABS Change -66

Narrative:

High-sensitivity Troponin I (hsTnI) assay can reliably detect low troponin concentrations relative to conventional troponin assays. It is the preferred marker of myocardial necrosis as recommended by the Fourth Universal Definition of Myocardial Infarction Guidelines.

The diagnosis of acute myocardial infarction (AMI) is made based on a rise or fall of troponin with at least one measurement exceeding the laboratory's upper limit of normal(indicating myocardial injury), in the context of reasonable suspicion for coronary ischemia (e.g. typical symptoms, changes on ECG, evidence for loss of myocardial function or demonstration of obstructive coronary artery disease).

NOTE: Although abnormal hsTnI values reflect INJURY to myocardial cells, an elevated hsTnI does not indicate the CAUSE of injury (i.e. ischemia versus non-ischemic disease).

In some cases, myocardial injury is chronic and relatively stable such that hsTnI values remain elevated but do not change substantially over hours to days (examples include chronic kidney disease, heart failure or advanced patient age).

When determining whether there has been a significant rise or fall of troponin on serial sampling, absolute change in troponin concentration has greater diagnostic accuracy for AMI than relative change criteria. A change of >7 ng/L over a 2-hour interval or a change of >10 ng/L over a 3-hour interval is suggested as a significant change. If the initial hsTnI is below or slightly above the 99th percentile upper reference limit, a 50% change from the baseline is considered significant. If the initial hsTnI is above the 99th percentile upper reference limit, a 20% change from the baseline value is considered significant.

Anti-Xa (Heparin Level) [560794889] (Abnormal) Collected: 12/04/25 0724

Specimen: Blood Updated: 12/04/25 0831

Heparin Anti-Xa 0.74 IU/mL

Basic Metabolic Panel [560774477] (Abnormal) Collected: 12/04/25 0421

Specimen: Blood Updated: 12/04/25 0505

Sodium 141 mmol/L

Potassium 3.6 mmol/L

Chloride 106 mmol/L

CO2 26.0 mmol/L

BUN 8.0 mg/dL

Creatinine 0.80 mg/dL

Glucose 93 mg/dL

Calcium 9.2 mg/dL

Glomerular Filtration Rate 79.4 mL/min/1.73m²

Anion Gap 9.0 mmol/L

BUN/Creatinine Ratio 10.0

CBC w/Differential [560774478] (Abnormal) Collected: 12/04/25 0421

Specimen: Blood Updated: 12/04/25 0458

WBC 4.90 10³/uL

RBC 3.96 10⁶/uL

Hemoglobin 11.8 g/dL

Hematocrit 34.9 %

MCV 87.9 fL

MCH 29.7 pg

MCHC 33.7 g/dL
RDW 14.7 %
Platelets 223 10³/uL
MPV 8.9 fL
Neutrophils (Relative) 47.1 %
Lymphocytes (Relative) 40.9 %
Monocytes (Relative) 9.4 %
Eosinophils (Relative) 2.1 %
Basophils (Relative) 0.5 %
Neutrophils (Absolute) 2.3 10³/uL
Lymphocytes (Absolute) 2.0 10³/uL
Monocytes (Absolute) 0.5 10³/uL
Eosinophils (Absolute) 0.1 10³/uL
Basophils (Absolute) 0.0 10³/uL
Hemoglobin A1c [560794865] (Normal) Collected: 12/04/25 0421
Specimen: Blood Updated: 12/04/25 0458
Hgb A1c 5.4 %
Lipid panel [560794866] (Abnormal) Collected: 12/04/25 0421
Specimen: Blood Updated: 12/04/25 0507
Cholesterol 198 mg/dL
Triglycerides 48 mg/dL
HDL 74.0 mg/dL
LDL CHOLESTEROL CALCULATED 114 mg/dL
Cardiac Risk 2.7 RATIO
VLDL 9.6 mg/dL
Troponin I [560794868] (Abnormal) Collected: 12/04/25 0421
Specimen: Blood Updated: 12/04/25 0521
HS Troponin I 1,366 ng/L
HS Troponin I % Change -13 %
HS Troponin I ABS Change -208

Narrative:

High-sensitivity Troponin I (hsTnI) assay can reliably detect low troponin concentrations relative to conventional troponin assays. It is the preferred marker of myocardial necrosis as recommended by the Fourth Universal Definition of Myocardial Infarction Guidelines.

The diagnosis of acute myocardial infarction (AMI) is made based on a rise or fall of troponin with at least one measurement exceeding the laboratory's upper limit of normal (indicating myocardial injury), in the context of reasonable suspicion for coronary ischemia (e.g. typical symptoms, changes on ECG, evidence for loss of myocardial function or demonstration of obstructive coronary artery disease).

NOTE: Although abnormal hsTnI values reflect INJURY to myocardial cells, an elevated hsTnI does not indicate the CAUSE of injury (i.e. ischemia versus non-ischemic disease).

In some cases, myocardial injury is chronic and relatively stable such that hsTnI values remain elevated but do not change substantially over hours to days (examples include chronic kidney disease, heart failure or advanced patient age).

When determining whether there has been a significant rise or fall of troponin on serial sampling, absolute change in troponin concentration has greater diagnostic accuracy for AMI than relative change criteria. A change of >7 ng/L over a 2-hour interval or a change of >10 ng/L over a 3-hour interval is suggested as a significant change. If the initial hsTnI is below or slightly above the 99th percentile upper reference limit, a 50% change from the baseline is considered significant. If the initial hsTnI is above the 99th percentile upper reference limit, a 20% change from the baseline value is considered significant.

B-TYPE NATRIURETIC PEPTIDE [560794869] (Normal) Collected: 12/04/25 0421
Specimen: Blood Updated: 12/04/25 0504
BNP 75.47 pg/mL
Magnesium [560794870] (Normal) Collected: 12/04/25 0421

Specimen: Blood Updated: 12/04/25 0505
Magnesium 1.90 mg/dL
Procalcitonin Test [560794874] (Normal) Collected: 12/04/25 0421
Specimen: Blood Updated: 12/04/25 0513
Procalcitonin <0.04 ng/mL
TSH Rfx on Abnormal to Free T4 [560794876] (Normal) Collected: 12/04/25 0421
Specimen: Blood Updated: 12/04/25 0507
TSH 1.342 uIU/mL
Anti-Xa (Heparin Level) [560794884] (Normal) Collected: 12/04/25 0421
Specimen: Blood Updated: 12/04/25 0500
Heparin Anti-Xa 0.58 IU/mL
SARS-CoV-2/Influenza/RSV Molecular Assay [560794875] (Normal) Collected: 12/04/25 0249
Specimen: Nasopharyngeal Swab Updated: 12/04/25 0629
SARS-CoV-2 by NAA Negative
Influenza A Negative
Influenza B Negative
Respiratory Syncytial Virus Negative
Anti-Xa (Heparin Level) [470194968] (Normal) Collected: 12/03/25 2021
Specimen: Blood Updated: 12/03/25 2043
Heparin Anti-Xa 0.56 IU/mL
TROPONIN I [470194971] (Abnormal) Collected: 12/03/25 2021
Specimen: Blood Updated: 12/03/25 2100
HS Troponin I 1,574 ng/L

Narrative:

High-sensitivity Troponin I (hsTnI) assay can reliably detect low troponin concentrations relative to conventional troponin assays. It is the preferred marker of myocardial necrosis as recommended by the Fourth Universal Definition of Myocardial Infarction Guidelines.

The diagnosis of acute myocardial infarction (AMI) is made based on a rise or fall of troponin with at least one measurement exceeding the laboratory's upper limit of normal (indicating myocardial injury), in the context of reasonable suspicion for coronary ischemia (e.g. typical symptoms, changes on ECG, evidence for loss of myocardial function or demonstration of obstructive coronary artery disease).

NOTE: Although abnormal hsTnI values reflect INJURY to myocardial cells, an elevated hsTnI does not indicate the CAUSE of injury (i.e. ischemia versus non-ischemic disease).

In some cases, myocardial injury is chronic and relatively stable such that hsTnI values remain elevated but do not change substantially over hours to days (examples include chronic kidney disease, heart failure or advanced patient age).

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ESTIMATED GFR [470194937] Collected: 12/03/25 1549
Updated: 12/03/25 1959
TROPONIN [470194938] (Abnormal) Collected: 12/03/25 1549
Updated: 12/03/25 1959
COMP METABOLIC PANEL [470194939] Collected: 12/03/25 1549
Updated: 12/03/25 1959
CBC WITH DIFFERENTIAL [470194940] (Abnormal) Collected: 12/03/25 1549
Updated: 12/03/25 1959
APTT [470194941] (Abnormal) Collected: 12/03/25 1547
Updated: 12/03/25 1959

Cardiovascular Studies:

EKG 12/3/25: NSR, anterolateral TWI

Monitor: NSR

TTE 7/10/24: The Ejection Fraction estimate is 65-70%

A variety of Doppler measurements indicate normal left ventricular diastolic function

The right ventricle is normal in size, thickness and function

There is moderate mitral regurgitation

Dual jets noted.

There is no pericardial effusion.

TEE:

Treadmill Exercise Stress Test:

SPECT:

Cardiac CT/MRI:

RHC/LHC 7/10/24: - Coronary angiogram via the right radial artery revealed mild, non-obstructive coronary artery disease in the LAD.

- OCT of the mid LAD stenosis revealed a calcified lesion with an MLA of 3.1 mm². No evidence of plaque rupture noted in the LAD.

- LVEDP was severely elevated to 48 mm Hg. LV gram not given due to high LVEDP.

- No gradient across the aortic valve.

Assessment and Plan:

Assessment:

70 y/o F still works, PMH of mild CAD (30% LAD stenosis), hx of NSTEMI/MINOCA due to Takutsubo 7/2024 (OCT showed no SCAD, it was probably Takutsubo vs vasospasm vs microvascular dysfunction), moderate MR, and HFpEF, whom cardiology is consulted for NSTEMI.

Patient did not present with CP, she has a lot of stress at work, she presented with dizziness and SOB.

She has not been taking ASA or statin or BB/CCB/Nitro or Lasix and instead taking supplements.

Labs showed HsT ~3000.

Plan:

1. NSTEMI
2. Hx of MINOCA 7/2024 (likely Takutsubo, but possibly spasm or microvascular dysfunction)
3. Mild LAD disease
4. Moderate MR with HFpEF

- I do not think this is due to obstructive CAD given LHC findings from 7/2024, this is likely recurrent Takustusbo or vasospasm or microvascular dysfunction.

- Continue heparin for 48 hours total.

- Start ASA 81 mg daily. Does not want statin at the moment.

- Obtain TTE.

- Based on TTE findings we will decide on coronary angiogram tomorrow vs just medical therapy (unclear if patient will adhere to full regimen as she prefers supplements over medications).

Thank you for giving us the opportunity to be a part of this patient's care. Please do not hesitate to contact us with any questions.

Mohammad Abu Sheikha, MD FACC

Board Certified Cardiologist

Electronically signed by Mohammad M Abu Sheikha, MD at 12/04/2025 9:57 AM PST

Nursing Notes - documented in this encounter

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[Regina Rayne Santos, RN - 12/05/2025 5:09 PM PST](#)

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[Hyacinth Ventura, RN - 12/05/2025 10:00 AM PST](#)

[Hyacinth Ventura, RN - 12/04/2025 4:25 PM PST](#)

[Hyacinth Ventura, RN - 12/04/2025 7:25 AM PST](#)

Hyacinth Ventura, RN - 12/05/2025 5:41 PM PST

Formatting of this note is different from the original.

Upon Discharge: Comments

Belongings returned to and verified by patient. ☒Yes ☐No*

Discharge clothing appropriate to weather. (clothing provided if needed) ☒Yes ☐No*

Verified discharge transportation and destination appropriate for patient needs ☒Yes ☐No*

DME arrangements completed as needed. ☒Yes ☐No*

Verified case management discharge planning complete. ☒Yes ☐No*

Discharge instructions given to patient/family. Time allowed for all questions to be answered. Patient/family verbalized understanding. ☒Yes ☐No*

Community resource list provided. ☒Yes ☐No*

Hospital wristband removed. ☒Yes ☐No*

Upon departure, patient escorted by staff or other transportation services. ☒Yes ☐No*

*Comments required for any "no" answers.

Documentation check list (ensure the following are documented in flowsheets).

Discharge assessment, vitals, and mobility ☒ Completed

Lines and drains: removal or reason to continue upon discharge ☒ Completed

☐ NA

Discharge wound photo (taken within 24 hours of D/C) uploaded, if applicable. ☐ Completed

☒ NA

Discharge paperwork and new medication explained, patient verbalized understanding. IV and wristbands removed.

Patient and all belongings wheeled down to ride.

Discharged instructions given and stat seal removed

1742 discharged home via wheelchair escorted by CNA Ingrid

Electronically signed by Hyacinth Ventura, RN at 12/05/2025 5:43 PM PST

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Regina Rayne Santos, RN - 12/05/2025 5:09 PM PST

Formatting of this note might be different from the original.

Cardiac Rehab referral explained to patient. Time allowed for all questions to be answered. Patient verbalized understanding. Cardiac Rehab Referral faxed.

Electronically signed by Regina Rayne Santos, RN at 12/05/2025 5:09 PM PST

[Back to top of Nursing Notes](#)

Hyacinth Ventura, RN - 12/05/2025 4:19 PM PST

Formatting of this note might be different from the original.

Aware Dr. Nagam that post-op vitals after post heart cath is atleast 4 hours due at 1900hr but as per patient she will not get a ride after that or she will stay for a night and will leave FOR TOMORROW. She said to contact Tracy from cardiology or case management. Aware Charge RN Regina.

1623 Contacted Tracy from Cardiology and said the above concern, She said she's ok with that, informed her , the site is good, no bleeding, able to urinate independently without dizziness. Charge RN Regina aware. Facilitated discharged

Electronically signed by Hyacinth Ventura, RN at 12/05/2025 5:59 PM PST

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Hyacinth Ventura, RN - 12/05/2025 2:53 PM PST

Formatting of this note might be different from the original.

Went back from cath lab on room air with TR band on.

Electronically signed by Hyacinth Ventura, RN at 12/05/2025 3:12 PM PST

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Hyacinth Ventura, RN - 12/05/2025 10:00 AM PST

Formatting of this note might be different from the original.

Patient expressed concerns , to meeting his son with a service dog to the lobby around 1130hr as tentative time of arrival, already aware the PA, said it was ok as long the nurse is beside her. Talked with Charge RN Regina, said to consult also with Dr. Nagam. Already informed the patient that service dogs can come up but it his son who just wants in the lobby.

1200 Dr. Nagam made rounds and expressed above concerns, agreed as long as the she will be with this RN and will just be quick

1211 the patient's son is available, went to the lobby, patient is ambulatory, no dizziness, no chest pain and converse for a little bit, then Charge RN Regina, came to informed as that cath lab nurses were already upstairs to pick her up.

1215 Came back to the room and Cath lab nurses were available to picked her up

1220 went to cath lab

Electronically signed by Hyacinth Ventura, RN at 12/05/2025 3:39 PM PST

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Hyacinth Ventura, RN - 12/04/2025 4:25 PM PST

Formatting of this note might be different from the original.

Tracy from Cardiology aware that patient's ECHO results in and also patient is expecting someone will explain the results to her. She said Dr. Abu will talk to her tomorrow.Relayed to the patient.

Electronically signed by Hyacinth Ventura, RN at 12/04/2025 5:57 PM PST

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Hyacinth Ventura, RN - 12/04/2025 7:25 AM PST

Formatting of this note might be different from the original.

Assuming care of patient at this time. SBAR report received from Antone Reno, RN. Pt awake in bed, no s/s of distress. A+Ox4, bed alarm is on, call light within reach. hourly rounding in place. Bed in lowest position, wheels locked.

Electronically signed by Hyacinth Ventura, RN at 12/04/2025 1:27 PM PST

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ED Notes - documented in this encounter

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[Rebekah Jackson, EMT - 12/04/2025 2:30 AM PST](#)

[Saul Lachino, RN - 12/04/2025 2:19 AM PST](#)

[Saul Lachino, RN - 12/03/2025 10:40 PM PST](#)

[Daniel B Van Bibber, MD - 12/03/2025 9:35 PM PST](#)

[Saul Lachino, RN - 12/03/2025 8:27 PM PST](#)

Rebekah Jackson, EMT - 12/04/2025 2:30 AM PST

Formatting of this note might be different from the original.

Called tele, Spoke to Rubelle and confirmed bed 463 is ready for transport.

Electronically signed by Rebekah Jackson, EMT at 12/04/2025 2:30 AM PST

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Saul Lachino, RN - 12/04/2025 2:19 AM PST

Formatting of this note is different from the original.

EMERGENCY DEPARTMENT TO FLOOR REPORT

Admitting Diagnosis:

Final diagnoses:

NSTEMI (non-ST elevated myocardial infarction) (CMS/HCC)

Patient Came From: arrived from: Home

Hx of Present Illness:

Chief Complaint

Patient presents with

Dizziness

Pt BIBA with CC of SOB and dizziness which began earlier today and has since has resolved. Per EMS Pt had a troponin at 2000 PTA.

Shortness of Breath

Pertinent Past Medical History:

Past Medical History:

Diagnosis Date

Broken heart syndrome

Interstitial cystitis

Past Surgical History:

Procedure Laterality Date

CYSTOSCOPY

DILATION AND CURETTAGE OF UTERUS

LAPAROSCOPY FOR ECTOPIC PREGNANCY

PR ARTHRP ACETBLR/PROX FEM PROSTC AGRFT/ALGRFT Right 3/4/2025

Procedure: RIGHT TOTAL HIP ARTHROPLASTY, DIRECT ANTERIOR APPROACH; Surgeon: Jessell M Owens, MD;

Location: NV SMRMC OR; Service: Orthopedics Surgery

SHOULDER SURGERY Right

Code Status: Full Code

Isolation/Infection: No active isolations

Allergies:

Allergies as of 12/03/2025 - Reviewed 12/03/2025

Allergen Reaction Noted

Citrus 02/28/2025
Codeine GI Intolerance 02/28/2025
Coffee flavor [flavoring agent] 02/28/2025
Gluten meal 02/28/2025
Vinegar [acetic acid] 02/28/2025

PIV: 20 Left Arm

Ambulation: Ambulates: standby assistance

Resp Status/ Supplemental Oxygen needs: RA

LOC/Mentation/GCS: A&O 4, GCS 15

Cardiac Rhythm: SR

Patient Behaviors: Calm and cooperative

Family Dynamics: N/A

Head to Toe Findings: Neurological
Neuro (WDL): Exceptions to WDL (Dizzines PTA)

Glasgow Coma Scale
Eye Opening: Spontaneous
Best Verbal Response: Oriented
Best Motor Response: Obeys commands
Glasgow Coma Scale Score: 15

Cardiac
Cardiac (WDL): Within Defined Limits

Peripheral Vascular
Peripheral Vascular (WDL): Within Defined Limits

Integumentary
Integumentary (WDL): Within Defined Limits

Braden Scale
Sensory Perceptions: No impairment
Moisture: Rarely moist
Activity: Walks frequently
Mobility: No limitation
Nutrition: Adequate
Friction and Shear: No apparent problem
Braden Scale Score: 22

Musculoskeletal
Musculoskeletal (WDL): Within Defined Limits

Gastrointestinal
Gastrointestinal (WDL): Within Defined Limits

Genitourinary
Genitourinary (WDL): Within Defined Limits

Psychosocial
Psychosocial (WDL): Within Defined Limits

Recent Vitals: Blood pressure (!) 142/80, pulse 65, temperature 97.9 °F (36.6 °C), temperature source Oral, resp. rate 13, height 5' 6" (1.676 m), weight 76.6 kg (168 lb 14 oz), SpO2 94%.
Vitals:

Recent Labs:
Labs Reviewed
TROPONIN I - Abnormal
Result Value
HS Troponin I 1,574 (*)

Narrative:
High-sensitivity Troponin I (hsTnI) assay can reliably detect low troponin concentrations relative to conventional troponin assays. It is the preferred marker of myocardial necrosis as recommended by the Fourth Universal Definition of Myocardial Infarction Guidelines.

The diagnosis of acute myocardial infarction (AMI) is made based on a rise or fall of troponin with at least one measurement exceeding the laboratory's upper limit of normal(indicating myocardial injury), in the context of reasonable suspicion for coronary ischemia (e.g. typical symptoms, changes on ECG, evidence for loss of myocardial function or demonstration of obstructive coronary artery disease).

NOTE: Although abnormal hsTnI values reflect INJURY to myocardial cells, an elevated hsTnI does not indicate the CAUSE of injury (i.e. ischemia versus non-ischemic disease).

In some cases, myocardial injury is chronic and relatively stable such that hsTnI values remain elevated but do not change substantially over hours to days (examples include chronic kidney disease, heart failure or advanced patient age).

When determining whether there has been a significant rise or fall of troponin on serial sampling, absolute change in troponin concentration has greater diagnostic accuracy for AMI than relative change criteria. A change of >7 ng/L over a 2-hour interval or a change of >10 ng/L over a 3-hour interval is suggested as a significant change. If the initial hsTnI is below or slightly above the 99th percentile upper reference limit, a 50% change from the baseline is

considered significant. If the initial hsTnl is above the 99th percentile upper reference limit, a 20% change from the baseline value is considered significant.

ANTI-XA - Normal

Heparin Anti-Xa 0.56

ANTI-XA

ANTI-XA

ANTI-XA

Imaging results:

No results found.

Meds given in ED:

Medications As of 12/03/25 2240

heparin 25000 units in d5w 250 mL (Units/kg/hr) Total dose: Not documented* Dosing weight: 76.6

*Total volume has not been documented. View each administration to see the amount administered.

Date/Time Rate/Dose/Volume Action Route Admin User

12/03/25 2219 12 Units/kg/hr - 9.2 mL/hr New Bag Intravenous Saul Lachino, RN
Destiny Linnell, RN

Orders Not completed in ED: ER orders complete

Call back to ED at x3188 for further questions

Electronically signed by Saul Lachino, RN at 12/04/2025 2:19 AM PST

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Saul Lachino, RN - 12/03/2025 10:40 PM PST

Formatting of this note is different from the original.

EMERGENCY DEPARTMENT TO FLOOR REPORT

Admitting Diagnosis:

Final diagnoses:

NSTEMI (non-ST elevated myocardial infarction) (CMS/HCC)

Patient Came From: arrived from: Home

Hx of Present Illness:

Chief Complaint

Patient presents with

Dizziness

Pt BIBA with CC of SOB and dizziness which began earlier today and has since has resolved. Per EMS Pt had a troponin at 2000 PTA.

Shortness of Breath

Pertinent Past Medical History:

Past Medical History:

Diagnosis Date

Broken heart syndrome

Interstitial cystitis

Past Surgical History:

Procedure Laterality Date

CYSTOSCOPY

DILATION AND CURETTAGE OF UTERUS

LAPAROSCOPY FOR ECTOPIC PREGNANCY

PR ARTHRP ACETBLR/PROX FEM PROSTC AGRFT/ALGRFT Right 3/4/2025

Procedure: RIGHT TOTAL HIP ARTHROPLASTY, DIRECT ANTERIOR APPROACH; Surgeon: Jessell M Owens, MD;

Location: NV SMRMC OR; Service: Orthopedics Surgery

SHOULDER SURGERY Right

Code Status: Full Code

Isolation/Infection: No active isolations

Allergies:

Allergies as of 12/03/2025 - Reviewed 12/03/2025

Allergen Reaction Noted

Citrus 02/28/2025

Codeine GI Intolerance 02/28/2025

Coffee flavor [flavoring agent] 02/28/2025

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PIV: 20 Left Arm

Ambulation: Ambulates: standby assistance

Resp Status/ Supplemental Oxygen needs: RA

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Peripheral Vascular (WDL): Within Defined Limits

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Integumentary (WDL): Within Defined Limits

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Sensory Perceptions: No impairment

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Activity: Walks frequently

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Friction and Shear: No apparent problem

Braden Scale Score: 22

Musculoskeletal

Musculoskeletal (WDL): Within Defined Limits

Gastrointestinal

Gastrointestinal (WDL): Within Defined Limits

Genitourinary

Genitourinary (WDL): Within Defined Limits

Psychosocial

Psychosocial (WDL): Within Defined Limits

Recent Vitals:

Vitals:

12/03/25 2006 12/03/25 2030 12/03/25 2100 12/03/25 2130

BP: (!) 134/56 138/70 114/74 121/67

Pulse: 66 68 64 64

Resp: 14 21 12 14

Temp: 97.9 °F (36.6 °C)

TempSrc: Oral

SpO2: 95% 98% 96% 97%

Weight: 76.6 kg (168 lb 14 oz)

Height: 5' 6" (1.676 m)

Recent Labs:

Labs Reviewed

TROPONIN I - Abnormal

Result Value

HS Troponin I 1,574 (*)

Narrative:

High-sensitivity Troponin I (hsTnI) assay can reliably detect low troponin concentrations relative to conventional troponin assays. It is the preferred marker of myocardial necrosis as recommended by the Fourth Universal Definition of Myocardial Infarction Guidelines.

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In some cases, myocardial injury is chronic and relatively stable such that hsTnI values remain elevated but do not change substantially over hours to days (examples include chronic kidney disease, heart failure or advanced patient age).

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ANTI-XA - Normal

Heparin Anti-Xa 0.56

ANTI-XA

ANTI-XA

ANTI-XA

Imaging results:

No results found.

Meds given in ED:

Medications As of 12/03/25 2240

heparin 25000 units in d5w 250 mL (Units/kg/hr) Total dose: Not documented* Dosing weight: 76.6

*Total volume has not been documented. View each administration to see the amount administered.

Date/Time Rate/Dose/Volume Action Route Admin User

12/03/25 2219 12 Units/kg/hr - 9.2 mL/hr New Bag Intravenous Saul Lachino, RN
Destiny Linnell, RN

Orders Not completed in ED: ER orders complete

Call back to ED at x3188 for further questions

Electronically signed by Saul Lachino, RN at 12/03/2025 10:41 PM PST

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Daniel B Van Bibber, MD - 12/03/2025 9:35 PM PST

Formatting of this note is different from the original.

Emergency Department Provider Note. Enc: 12/3/2025 7:58 PM

Michelle Caruso MRN: 116253192 Acct# 730000366346 SMRMC EMERGENCY

HPI

Chief Complaint

Patient presents with

- Dizziness

Pt BIBA with CC of SOB and dizziness which began earlier today and has since has resolved. Per EMS Pt had a troponin at 2000 PTA.

- Shortness of Breath

Patient has been outside facility. She was seen earlier today there was some symptoms of some shortness of breath throughout the day. She has been some lightheadedness like she might pass out at times. She has a dressing to be managed in nature, she would notice it more when she is active versus at rest. Was not associated with the areas has been however she has been putting up with a sent her to the emergency room he had an outside facility where she was vomiting do not have troponin greater than 2000. She is transferred here for Cardiology availability. Currently she is asymptomatic. She has no prior history of cardiac disease except she said she had "broken heart syndrome" in the past and has been for the past away.

Denies any fevers, cough, sore throat, abdominal pain, chest pain

REVIEW OF SYSTEMS

Review of Systems

Other ROS as per HPI

PMH

Medications:

Previous Medications

No medications on file

PMH:

Past Medical History:

Diagnosis Date

- Broken heart syndrome
- Interstitial cystitis

Past Medical history reviewed with the patient by me and documented in the nursing record

PSHX:

Past Surgical History:

Procedure Laterality Date

- CYSTOSCOPY
- DILATION AND CURETTAGE OF UTERUS
- LAPAROSCOPY FOR ECTOPIC PREGNANCY
- PR ARTHRP ACETBLR/PROX FEM PROSTC AGRFT/ALGRFT Right 3/4/2025

Procedure: RIGHT TOTAL HIP ARTHROPLASTY, DIRECT ANTERIOR APPROACH; Surgeon: Jessell M Owens, MD;

Location: NV SMRMC OR; Service: Orthopedics Surgery

- SHOULDER SURGERY Right

Past Surgical history reviewed with the patient by me and documented in the nursing record

Social:

Social History

Tobacco Use

- Smoking status: Never
- Smokeless tobacco: Never

Vaping Use

- Vaping status: never used

Substance Use Topics

- Alcohol use: Yes

Comment: 5 per year

- Drug use: Never

Social history reviewed with the patient by me and documented in the nursing record

Family:

Family History

Problem Relation Age of Onset

- Cancer Father
- Heart disease Father

Family history reviewed with the patient by me and documented in the nursing record

Allergies: She is allergic to citrus, codeine, coffee flavor [flavoring agent], gluten meal, and vinegar [acetic acid].

PHYSICAL EXAMINATION

Vitals:

12/03/25 2006 12/03/25 2030 12/03/25 2100

BP: (!) 134/56 138/70 114/74

Pulse: 66 68 64

Resp: 14 21 12

Temp: 97.9 °F (36.6 °C)

TempSrc: Oral

SpO2: 95% 98% 96%

Weight: 76.6 kg (168 lb 14 oz)

Height: 5' 6" (1.676 m)

Physical Exam

Vitals reviewed.

Constitutional:

General: She is not in acute distress.

Appearance: She is not ill-appearing.

HENT:

Head: Normocephalic.

Nose: Nose normal.

Mouth/Throat:

Mouth: Mucous membranes are moist.

Eyes:

Conjunctiva/sclera: Conjunctivae normal.

Pupils: Pupils are equal, round, and reactive to light.

Cardiovascular:

Rate and Rhythm: Normal rate.

Pulses: Normal pulses.

Pulmonary:

Effort: Pulmonary effort is normal.

Breath sounds: Normal breath sounds.

Abdominal:

General: There is no distension.

Palpations: Abdomen is soft.

Tenderness: There is no abdominal tenderness.

Musculoskeletal:

General: No swelling or deformity.

Cervical back: Normal range of motion and neck supple.

Skin:

General: Skin is warm and dry.

Findings: No rash.

Neurological:

Mental Status: She is alert and oriented to person, place, and time.

Motor: No weakness.

Psychiatric:

Mood and Affect: Mood normal.

Data

EKG (if indicated): Sinus rhythm. 62 beats per minute. Normal P waves, normal narrow complex QRS. Inverted T-waves in aVL and lead one, but no other acute ischemia.

EKG reviewed by me including rate, rhythm, intervals, ST changes and is documented in the medical record when indicated.

Lab Results (if indicated):

Labs Reviewed

TROPONIN I - Abnormal

Result Value

HS Troponin I 1,574 (*)

Narrative:

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ANTI-XA - Normal

Heparin Anti-Xa 0.56

ANTI-XA

Imaging Results (if indicated):

Imaging Results

None

Labs & imaging studies/reports reviewed and interpreted independently by me.

ED COURSE

Orders:

Orders Placed This Encounter

Procedures

- Anti-Xa (Heparin Level)
- TROPONIN I
- Nursing Communication

ED Medications Administered (if applicable):

Medications

PHARMACY TO DOSE HEPARIN 1 Consult (has no administration in time range)

Procedures (if indicated):

Procedures

Course:

ED Course as of 12/03/25 2150

Wed Dec 03, 2025

2149 Case discussed with Dr. Carrea cardiology, agreed with plan of heparin and admission, he will see pt in the hospital. [DV]

ED Course User Index

[DV] Daniel B Van Bibber, MD

MEDICAL DECISION MAKING

Patient has a transfer from outside facility with an NSTEMI.

We will continue heparin, consult cardiology and admit to the hospital.

Severity/Risk: The patient's overall risk, complexity and symptoms appear to be:

SEVERITY/RISK: High

COMPLEXITY: High

SYMPTOMS: Moderate

ACUITY: Acute

There was 1 or more acute or chronic illness at this emergency department visit.

Diagnostics: Laboratory (if applicable) and imaging (if applicable) studies were independently reviewed and interpreted by me. These include EKG and labs.

Medical Management (if applicable): Pt required emergent parenteral medications including heparin therapy and needed continuous cardiorespiratory monitoring given the potential of cardiorespiratory side effects of these medications.

Old records. I reviewed the patient's prior medical record from from outside facility from earlier today where she was initially seen

Comorbidities (has a past medical history of Broken heart syndrome and Interstitial cystitis.) reviewed and factored into the workup of the patient's case.

Consults:

Cardiology as above in ED course.

Consulted with hospital physician for admission to the hospital due to critical illness of NSTEMI.

Critical Care Time (if applicable):

CRITICAL CARE TIME:

Upon my evaluation, this patient had a high probability of imminent or life-threatening deterioration due to direct risk to the cardiovascular system, which required my direct attention, intervention, and personal management.

I have personally provided 35 min of critical care time exclusive of time spent on separately billable procedures. Time includes review of laboratory data, radiology results, discussion with consultants, and close monitoring for potential decompensation. Multiple Interventions were performed and serial evaluations as documented above.

Incidental findings:

Disposition:

Admit

Clinical Impression:

1. NSTEMI (non-ST elevated myocardial infarction) (CMS/HCC)

New Prescriptions

No medications on file

Patient to follow up with:

Incidental Findings:

This note was partially dictated utilizing Dragon nuance technology. Effort was made to proofread the document prior to signing, however grammatical and/or contextual errors are still possible and may be present after signing

Entered by Daniel B Van Bibber, acting as scribe for Dr. Van Bibber MD

Daniel B Van Bibber, MD

12/03/25 2140

Electronically signed by Daniel B Van Bibber, MD at 12/03/2025 9:40 PM PST

[Back to top of ED Notes](#)

Saul Lachino, RN - 12/03/2025 8:27 PM PST

Formatting of this note might be different from the original.

Per MD hold infusion of heparin from sending facility at this time pending anti XA results.

Electronically signed by Saul Lachino, RN at 12/03/2025 8:28 PM PST

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Miscellaneous Notes - documented in this encounter

Table of Contents for Miscellaneous Notes

[CDI Query - Manvi R Nagam, MD - 12/05/2025 5:53 PM PST](#)

[Plan of Care - Mohammad M Abu Sheikha, MD - 12/05/2025 2:51 PM PST](#)

[Case Management - Vanessa Capriglione - 12/05/2025 12:01 PM PST](#)

[Plan of Care - Hyacinth Ventura, RN - 12/05/2025 11:00 AM PST](#)

[Plan of Care - Antone Reno, RN - 12/05/2025 6:59 AM PST](#)

[Case Management - Vanessa Capriglione - 12/04/2025 2:55 PM PST](#)

[Case Management - Vanessa Capriglione - 12/04/2025 2:52 PM PST](#)

[Case Management - Vanessa Capriglione - 12/04/2025 2:52 PM PST](#)

[Case Management - Vanessa Capriglione - 12/04/2025 11:36 AM PST](#)

[Plan of Care - Hyacinth Ventura, RN - 12/04/2025 11:06 AM PST](#)

[Plan of Care - Antone Reno, RN - 12/04/2025 6:49 AM PST](#)

[ED Triage Note - Saul Lachino, RN - 12/03/2025 8:08 PM PST](#)

CDI Query - Manvi R Nagam, MD - 12/05/2025 5:53 PM PST

Formatting of this note is different from the original.

PHYSICIAN CLARIFICATION

Dear Dr: Manvi R Nagam, MD

Date: 12/10/2025

Coder/CDS: VEGULLA, VISALAKSHMI

Phone #:

QUERY

Exercise your independent professional judgment when responding to this query. Questions asked do not imply that a particular answer is desired or expected. We greatly appreciate your clarification on this issue.

Clinical documentation states:

H&P Shantz, Allison E NP 12/03/25

A 70-year-old female presented with dizziness and dyspnea since this morning, unrelated to activity. She experienced stomach cramps and diarrhea yesterday, now asymptomatic.,

Assessment: Non STEMI, possible right middle lung pneumonia, dyslipidemia.

Progress Notes Nagam, Manvi R MD 12/04/25

Assessment: NSTEMI, History of Takotsubo's cardiomyopathy, Dyslipidemia

Discharge Summary Nagam, Manvi R MD 12/05/25

Assessment: NSTEMI-likely secondary to coronary vasospasm, Dyslipidemia

Clinical findings show:

PSI index: class 2

Vitals: TEMP HR RR (SBP/DBP) SPO2

3-Dec 97.9 °F 66 14 (134/56) 95

4-Dec 97.7 °F 59 18 (144/80) 96

Labs: WBC Bands

4-Dec 4.9 2.3

5-Dec 4.6 2.1

Radiology:

Xr-chest Ciccarelli, Andrew M MD 12/04/25

No acute chest disease.

Medications:

IV Ceftriaxone 1 G In Dextrose 5 % 50 MI Ivpb Add-Ease 12/03-12/10;

IV Doxycycline 100 Mg In Dextrose 5 % 100 MI Ivpb Add-Ease 12/03-12/10;

Based on the clinical presentation, please clarify the patient's condition:

☐ Gram negative pneumonia

☐ Gram positive pneumonia

☐ Lobar/basilar pneumonia

☐ Other bacterial pneumonia (please specify): _____

☐ Other explanation of clinical findings. Please specify: Patient was initially treated with empirical antibiotics for possible pneumonia, however chest x-ray did not show findings of pneumonia, patient had no fever or leukocytosis, procalcitonin was within normal limits. _____

Present on Admission: No (N)

Please also document response in your Progress Notes and/or Discharge Summary and indicate if the condition was present on admission.

PATIENT NAME:Michelle Caruso MR#:116253192

DOB:2/19/1955 HAR#:730000366346

AGE/SEX:70 y.o., female ADMIT DATE:12/3/2025 7:58 PM

Reference:

Pneumonia severity index

<https://reference.medscape.com/calculator/248/pneumonia-severity-index-port-score>

Electronically signed by Manvi R Nagam, MD at 12/15/2025 5:32 AM PST

[Back to top of Miscellaneous Notes](#)

Plan of Care - Mohammad M Abu Sheikha, MD - 12/05/2025 2:51 PM PST

Formatting of this note might be different from the original.

Cath showed no obstructive CAD. TTE showed no Takutsubo.

Recurrent MINOCA is likely in the setting of coronary vasospasm or microvascular dysfunction.

Recommend outpatient cardiac MRI for complete work-up.

Patient can go home today from cardiac standpoint on ASA 81 mg daily, Lipitor 40 mg daily, and Cardizem ER 30 mg daily.

Follow up will be arranged.

Electronically signed by Mohammad M Abu Sheikha, MD at 12/05/2025 2:53 PM PST

[Back to top of Miscellaneous Notes](#)

Case Management - Vanessa Capriglione - 12/05/2025 12:01 PM PST

Formatting of this note is different from the original.

12/05/25 1200

Discharge Planning Communication

Reason for Communication Clinical Team Communication

Discussion Summary Discussed in AM CM meeting. Per hospitalist, patient is pending cath lab and is not medically cleared at this time. May discharge home with no needs later today or tomorrow.

Plan of Care - Hyacinth Ventura, RN - 12/05/2025 11:00 AM PST

Formatting of this note might be different from the original.

Problem: Activity:

Goal: Mobility will improve

Outcome: Progressing

Problem: Cognitive:

Goal: Ability to state ways to decrease the risk of falls will improve

Outcome: Progressing

Problem: Nutritional:

Goal: Nutritional status will improve

Outcome: Progressing

Problem: Safety:

Goal: Ability to identify and utilize support systems that promote safety will improve

Outcome: Progressing

Goal: Will remain free from falls

Outcome: Progressing

Problem: Activity:

Goal: Ability to tolerate increased activity will improve

Outcome: Progressing

Problem: Cardiac:

Goal: Ability to achieve and maintain adequate cardiopulmonary perfusion will improve

Outcome: Progressing

Goal: Diagnostic test results will improve

Outcome: Progressing

Goal: Will show no evidence of cardiac dysrhythmia

Outcome: Progressing

Problem: Cognitive:

Goal: Knowledge of disease or condition will improve

Outcome: Progressing

Goal: Ability to identify changes in lifestyle to reduce recurrence of condition will improve

Outcome: Progressing

Problem: Coping:

Goal: Level of anxiety will decrease

Outcome: Progressing

Goal: Mental status will improve

Outcome: Progressing

Problem: Health Behavior:

Goal: Identification of resources available to assist in meeting health care needs will improve

Outcome: Progressing

Problem: Respiratory:

Goal: Levels of oxygenation will improve

Outcome: Progressing

Goal: Respiratory status will improve

Outcome: Progressing

Plan of Care - Antone Reno, RN - 12/05/2025 6:59 AM PST

Formatting of this note might be different from the original.

Problem: Activity:

Goal: Mobility will improve

Outcome: Progressing

Problem: Cognitive:

Goal: Ability to state ways to decrease the risk of falls will improve

Outcome: Progressing

Problem: Nutritional:

Goal: Nutritional status will improve

Outcome: Progressing

Problem: Safety:

Goal: Ability to identify and utilize support systems that promote safety will improve

Outcome: Progressing

Goal: Will remain free from falls

Outcome: Progressing

Problem: Activity:

Goal: Ability to tolerate increased activity will improve

Outcome: Progressing

Problem: Cardiac:

Goal: Ability to achieve and maintain adequate cardiopulmonary perfusion will improve

Outcome: Progressing

Goal: Diagnostic test results will improve

Outcome: Progressing

Goal: Will show no evidence of cardiac dysrhythmia

Outcome: Progressing

Problem: Cognitive:

Goal: Knowledge of disease or condition will improve

Outcome: Progressing

Goal: Ability to identify changes in lifestyle to reduce recurrence of condition will improve

Outcome: Progressing

Problem: Coping:

Goal: Level of anxiety will decrease

Outcome: Progressing

Goal: Mental status will improve

Outcome: Progressing

Problem: Health Behavior:

Goal: Identification of resources available to assist in meeting health care needs will improve

Outcome: Progressing

Problem: Respiratory:

Goal: Levels of oxygenation will improve

Outcome: Progressing

Goal: Respiratory status will improve

Outcome: Progressing

Electronically signed by Antone Reno, RN at 12/05/2025 6:59 AM PST

[Back to top of Miscellaneous Notes](#)

Case Management - Vanessa Capriglione - 12/04/2025 2:55 PM PST

Formatting of this note is different from the original.

12/04/25 1452

Ongoing Discharge Planning

DC Planning Charting type Follow up

Living Arrangements Children

Patient Arrived From? Private residence

Usual Residence Private residence

Support Systems Children

Patient Designated Caregiver Son Sebastian 808-278-8802

Consent Signed to Release Medical Information to Caregiver No

Patient Unable to Designate a Caregiver, Follow up Needed No

Patient Declined to Designate a Caregiver No

Assistance Needed Independent at baseline with no DME or o2. States that she has a cane and a walker at home but does not use them.

Current Durable Medical Equipment (DME) Walker;Other (comment)
(cane)

Recommended DME to Discharge Walker;Other (comment)
(cane)

Current Residence Private residence

of Steps at Residence 1

Home Care Services No

Patient expects to be discharged to: Private residence

Referral to Discharge Planning/Case Management Discharge Planning Needs

Additional Comments Demographics verified with the patient at bedside. Patient states that she still work full-time and is very independent. Does have her son for support. Denies previous HH or SNF. Able to drive. Son will transport at time of discharge. PCP: Sarah Maples, last seen a few months ago. DCP: Anticipate home with no needs.

Supporting Self-Management Post Stroke Patient/Family/Caregiver assessed for readiness, willingness, and ability to engage in or provide support for self-management.

IMM reviewed and understood.

Electronically signed by Vanessa Capriglione at 12/04/2025 2:56 PM PST

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Case Management - Vanessa Capriglione - 12/04/2025 2:52 PM PST

Formatting of this note is different from the original.

12/04/25 1300

Advance Directives (For Healthcare)

Advance Directive Patient has advance directive, copy not in chart

Advance Directive not in Chart Copy requested from other (Comment)

(Requested that a copy is brought in if possible.)

Electronically signed by Vanessa Capriglione at 12/04/2025 2:52 PM PST

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Case Management - Vanessa Capriglione - 12/04/2025 2:52 PM PST

Formatting of this note is different from the original.

12/04/25 1451

SDOH Needs Follow ups

Housing Stability Risk Patient/Family Declined Referral

Food Insecurity Needs Patient/Family Declined Referral

Utilities Risk Patient/Family Declined Referral

Financial Resource Strain Patient/Family Declined Referral
Transportation Needs Patient/Family Declined Referral
Intimate Partner Violence (Interpersonal Safety Risk) Patient/Family Declined Referral
Social Connections Patient/Family Declined Referral

Electronically signed by Vanessa Capriglione at 12/04/2025 2:52 PM PST

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Case Management - Vanessa Capriglione - 12/04/2025 11:36 AM PST

Formatting of this note is different from the original.

12/04/25 1100

Discharge Planning Communication

Reason for Communication Clinical Team Communication

Discussion Summary Discussed in AM CM meeting. Per hospitalist, patient is having an echo today and is not medically cleared at this time. DCP: TBD.

Electronically signed by Vanessa Capriglione at 12/04/2025 11:36 AM PST

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Plan of Care - Hyacinth Ventura, RN - 12/04/2025 11:06 AM PST

Formatting of this note might be different from the original.

Problem: Activity:

Goal: Mobility will improve

Outcome: Progressing

Problem: Cognitive:

Goal: Ability to state ways to decrease the risk of falls will improve

Outcome: Progressing

Problem: Nutritional:

Goal: Nutritional status will improve

Outcome: Progressing

Problem: Safety:

Goal: Ability to identify and utilize support systems that promote safety will improve

Outcome: Progressing

Goal: Will remain free from falls

Outcome: Progressing

Problem: Activity:

Goal: Ability to tolerate increased activity will improve

Outcome: Progressing

Problem: Cardiac:

Goal: Ability to achieve and maintain adequate cardiopulmonary perfusion will improve

Outcome: Progressing

Goal: Diagnostic test results will improve

Outcome: Progressing

Goal: Will show no evidence of cardiac dysrhythmia

Outcome: Progressing

Problem: Cognitive:

Goal: Knowledge of disease or condition will improve

Outcome: Progressing

Goal: Ability to identify changes in lifestyle to reduce recurrence of condition will improve

Outcome: Progressing

Problem: Coping:

Goal: Level of anxiety will decrease

Outcome: Progressing

Goal: Mental status will improve

Outcome: Progressing

Problem: Health Behavior:

Goal: Identification of resources available to assist in meeting health care needs will improve

Outcome: Progressing

Problem: Respiratory:

Goal: Levels of oxygenation will improve

Outcome: Progressing

Goal: Respiratory status will improve

Outcome: Progressing

Electronically signed by Hyacinth Ventura, RN at 12/04/2025 11:07 AM PST

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Plan of Care - Antone Reno, RN - 12/04/2025 6:49 AM PST

Formatting of this note might be different from the original.

Problem: Activity:

Goal: Mobility will improve

Outcome: Progressing

Problem: Cognitive:

Goal: Ability to state ways to decrease the risk of falls will improve

Outcome: Progressing

Problem: Nutritional:

Goal: Nutritional status will improve

Outcome: Progressing

Problem: Safety:

Goal: Ability to identify and utilize support systems that promote safety will improve

Outcome: Progressing

Goal: Will remain free from falls

Outcome: Progressing

Problem: Activity:

Goal: Ability to tolerate increased activity will improve

Outcome: Progressing

Problem: Cardiac:

Goal: Ability to achieve and maintain adequate cardiopulmonary perfusion will improve

Outcome: Progressing

Goal: Diagnostic test results will improve

Outcome: Progressing

Goal: Will show no evidence of cardiac dysrhythmia

Outcome: Progressing

Problem: Cognitive:

Goal: Knowledge of disease or condition will improve

Outcome: Progressing

Goal: Ability to identify changes in lifestyle to reduce recurrence of condition will improve

Outcome: Progressing

Problem: Coping:

Goal: Level of anxiety will decrease

Outcome: Progressing

Goal: Mental status will improve

Outcome: Progressing

Problem: Health Behavior:

Goal: Identification of resources available to assist in meeting health care needs will improve

Outcome: Progressing

Problem: Respiratory:

Goal: Levels of oxygenation will improve

Outcome: Progressing

Goal: Respiratory status will improve

Outcome: Progressing

Electronically signed by Antone Reno, RN at 12/04/2025 6:49 AM PST

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ED Triage Note - Saul Lachino, RN - 12/03/2025 8:08 PM PST

Formatting of this note is different from the original.

Chief Complaint

Patient presents with

Dizziness

Pt BIBA with CC of SOB and dizziness which began earlier today and has since has resolved. Per EMS Pt had a troponin at 2000 PTA.

Shortness of Breath

Vitals:

12/03/25 2006

BP: (!) 134/56

Pulse: 66

Resp: 14

Temp: 97.9 °F (36.6 °C)

SpO2: 95%

Electronically signed by Saul Lachino, RN at 12/03/2025 8:08 PM PST

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Plan of Treatment - documented as of this encounter

Not on file

ECG Results - documented in this encounter

ECG 12 Lead (12/03/2025 8:06 PM PST)

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			12/03/2025 8:06 PM PST	

Impressions

ECS - 12/03/2025 8:06 PM PST
St. Mary's Regional ED

Test Date:	2025-12-03	Test Time:	20:06:45
Pat Name:	MICHELLE CARUSO	Department:	4NE
Patient ID:	SMRMC-116253192	Room:	SMRMC 463
Gender:	F	Technician:	MBARBAGELATA
DOB:	1955-02-19	Requested By:	DANIEL VAN BIBBER
Order Number:	560794887SMRMC	Reading MD:	Brock Boscovich

Intervals		Measurements	
Rate:	66	Axis	
PR:	164	P:	53
QRSD:	92	QRS:	8
QT:	459	T:	111
QTc:	481		

Interpretive Statements

Sinus rhythm
Abnrm T, probable ischemia, anterolateral lds
No previous ECG available for comparison
Electronically signed on 12-06-2025 12:49:00 PST by Brock Boscovich

Narrative

Authorizing Provider	Result Type	Result Status
Daniel B Van Bibber MD	ECG ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
ECS			

Visit Diagnoses - documented in this encounter

Diagnosis

NSTEMI (non-ST elevated myocardial infarction) (CMS/HCC) - Primary
Acute myocardial infarction, subendocardial infarction, episode of care unspecified
NSTEMI (non-ST elevated myocardial infarction) (CMS/HCC)
Acute myocardial infarction, subendocardial infarction, episode of care unspecified

Admitting Diagnoses - documented in this encounter

Diagnosis

NSTEMI (non-ST elevated myocardial infarction) (CMS/HCC)
Acute myocardial infarction, subendocardial infarction, episode of care unspecified

Administered Medications - documented in this encounter

Inactive Administered Medications - up to 3 most recent administrations

Medication Order	MAR Action	Action Date	Dose	Rate	Site
acetaminophen (TYLENOL) tablet 650 mg 650 mg, Oral, Every 6 hours as needed, mild pain (1-3), moderate pain (4-6), headaches, Starting on Wed 12/3/25 at 2341, The max daily dose for patients less than 50 kg is 75 mg/kg/24 hours and for patients 50 kg or greater is 4000 mg/24 hours. Consider all source of Acetaminophen.					
artificial tears ophthalmic solution 1 drop 1 drop, Both Eyes, Every 1 hour as needed, dry eyes, Starting on Wed 12/3/25 at 2347	Given	12/04/2025 12:39 AM PST	1 drop		
aspirin chewable tablet 81 mg 81 mg, Oral, Daily, First dose on Thu 12/4/25 at 0945, Give with food	Given	12/05/2025 8:38 AM PST	81 mg		
	Given	12/04/2025 10:18 AM PST	81 mg		
atorvastatin (LIPITOR) tablet 40 mg 40 mg, Oral, At bedtime, First dose on Thu 12/4/25 at 2100	Given	12/04/2025 9:49 PM PST	40 mg		
bisacodyl (DULCOLAX) suppository 10 mg 10 mg, Rectal, Daily as needed, constipation, Starting on Wed 12/3/25 at 2339, If milk of magnesia, bisacodyl tablet, docusate, senna or GLYCOLAX ineffective or not ordered					
cefTRIAxone (ROCEPHIN) 1 g in dextrose 5 % 50 mL IVPB ADD-EASE 1 g, Intravenous, at 100 mL/hr, Every 24 hours, First dose on Wed 12/3/25 at 2345, For 7 days, Indication: CAP Pneumonia	New Bag	12/05/2025 12:00 AM PST	1 g	100 mL/hr	
	New Bag	12/04/2025 12:00 AM PST	1 g	100 mL/hr	
docusate sodium (COLACE) tablet/capsule 100 mg 100 mg, Oral, Daily as needed, constipation, if milk of magnesia and bisacodyl tablet ineffective or not ordered, Starting on Wed 12/3/25 at 2339					
doxycycline (VIBRAMYCIN) 100 mg in dextrose 5 % 100 mL IVPB ADD-EASE 100 mg, Intravenous, Administer over 60 Minutes, Every 12 hours, First dose on Wed 12/3/25 at 2345, For 7 days, Indication: CAP Pneumonia	New Bag	12/04/2025 11:10 PM PST	100 mg	100 mL/hr	
	New Bag	12/04/2025 12:01 PM PST	100 mg	100 mL/hr	
	New Bag	12/04/2025 2:05 AM PST	100 mg	100 mL/hr	

heparin 25000 units in d5w 250 mL
 12 Units/kg/hr $\times$ 76.6 kg (9.192 mL/hr, rounded to 9.2 mL/hr), Intravenous, Continuous, Starting on Wed 12/3/25 at 2145, Maximum initial rate: 1000 units/hr. * * *
BLACK BOX WARNING * * * SOME PRODUCTS CONTAIN BENZYL ALCOHOL AS A PRESERVATIVE; USE OF THESE PRODUCTS IS CONTRAINDICATED IN NEONATES., Initial Dose (units/kg/hr): 12, Maximum Initial Dose (units/hr): 1000, Anti-Xa <0.1 Maintenance Dose Adjustment (units/kg/hr): 4, Anti-Xa <0.1 Bolus Dose (units/kg): 50, Anti-Xa <0.1 Additional Instructions: Rebolus at 50 units/kg (maximum rebolus dose: 4000 units). Then INCREASE current dose by 4 units/kg/hr., Anti-Xa 0.1-0.19 Maintenance Dose Adjustment (units/kg/hr): 3, Anti-Xa 0.1-0.19 Bolus Dose (units/kg): 25, Anti-Xa 0.1-0.19 Additional Instructions: Rebolus at 25 units/kg (maximum rebolus dose: 4000 units). Then INCREASE current dose by 3 units/kg/hr., Anti-Xa 0.2-0.29 Maintenance Dose Adjustment (units/kg/hr): 2, Anti-Xa 0.2-0.29 Bolus Dose (units/kg): 25, Anti-Xa 0.2-0.29 Additional Instructions: Rebolus at 25 units/kg (maximum rebolus dose: 4000 units). Then INCREASE current dose by 2 units/kg/hr., Anti-Xa less than or equal to 0.29 Maximum Rebolus Dose (units): 4000, Anti-Xa 0.3-0.7 Maintenance Dose Adjustment (units/kg/hr): 0, Anti-Xa 0.3-0.7 Bolus Dose (units/kg): 0, Anti-Xa 0.3-0.7 Additional Instructions: Continue current infusion dose. No bolus needed. At therapeutic range., Anti-Xa 0.71-0.8 Maintenance Dose Adjustment (units/kg/hr): -1, Anti-Xa 0.71-0.8 Bolus Dose (units/kg): 0, Anti-Xa 0.71-0.8 Additional Instructions: DECREASING previous dose by 1 units/kg/hour., Anti-Xa 0.81-0.9 Maintenance Dose Adjustment (units/kg/hr): -2, Anti-Xa 0.81-0.9 Bolus Dose (units/kg): 0, Anti-Xa 0.81-0.9 Additional Instructions: DECREASING previous dose by 2 units/kg/hour., Anti-Xa 0.91-1 Maintenance Dose Adjustment (units/kg/hr): -3, Anti-Xa 0.91-1 Bolus Dose (units/kg): 0, Anti-Xa 0.91-1 Additional Instructions: Hold infusion for 1 hour. Then restart by DECREASING previous dose by 3 units/kg/hour., Anti-Xa >1 Maintenance Dose Adjustment (units/kg/hr): -4, Anti-Xa >1 Bolus Dose (units/kg): 0, Anti-Xa >1 Additional Instructions: Stop infusion and notify provider (Comment: repeat anti-Xa hourly until <0.7 then resume infusion at 4 units/kg/hr less than previously documented dose).

New Bag	12/05/2025 12:56 AM PST	11 Units/kg/hr	8.4 mL/hr
Rate/Dose Change	12/04/2025 4:20 PM PST	11 Units/kg/hr	8.4 mL/hr
Rate/Dose Verify	12/04/2025 4:18 PM PST	11 Units/kg/hr	8.4 mL/hr

Indications: Non-ST-Segment Elevation
Myocardial Infarction

hydrALAZINE (APRESOLINE) injection 10 mg
10 mg, Intravenous, Every 4 hours as
needed, SBP > 170 or DBP > 100, Starting on
Wed 12/3/25 at 2342

nitroglycerin (NITROSTAT) SL tablet 0.4 mg
0.4 mg, Sublingual, Every 5 minutes x3 as
needed, chest pain, Starting on Wed 12/3/25
at 2342, May administer up to 3 doses per
episode * * * DO NOT CRUSH OR CHEW * * *

ondansetron (ZOFTRAN) injection 4 mg
4 mg, Intravenous, Every 4 hours as needed,
nausea, vomiting, Starting on Wed 12/3/25 at
2339, If patient is NPO or unable to tolerate
PO

ondansetron (ZOFTRAN-ODT) disintegrating
tablet 4 mg
4 mg, Oral, Every 6 hours as needed, nausea,
vomiting, Starting on Wed 12/3/25 at 2339

polyethylene glycol (GLYCOLAX) powder 17 g
17 g, Oral, Daily as needed, constipation,
constipation if milk of magnesia, bisacodyl
tablet, docusate sodium and senna ineffective
or not ordered, Starting on Wed 12/3/25 at
2339

sodium chloride (NS) 0.9 % flush 3 mL
3 mL, Intravenous, Every 8 hours as needed,
line care, Starting on Fri 12/5/25 at 1503

Discontinued Medications - documented as of this encounter

Medication	Sig	Discontinue Reason	Start Date	End Date
aspirin 81 MG EC tablet	Take 1 tablet (81 mg total) by mouth daily		03/05/2025	12/05/2025

Historical Medications - added in this encounter

This list may reflect changes made after this encounter.

Medication	Sig	Dispense Quantity	Refills	Last Filled	Start Date	End Date
aspirin 81 MG EC tablet	Take 1 tablet (81 mg total) by mouth daily				03/05/2025	12/05/2025

Active and Recently Administered Medications - documented in this encounter

Legend

Given	Not Given	Canceled	Hold	Due	Other	
<u>Time</u>	<u>(Time)</u>	Entry	Time	Time	Time-	Times are shown in PST.
		Time	Time		Action	

Scheduled

Medication Order	12/03/2025	12/04/2025	12/05/2025
aspirin chewable tablet 81 mg 81 mg, Oral, Daily, First dose on Thu 12/4/25 at 0945, Give with food		1018	0838 1414-MAR Hold 1504-MAR Unhold
atorvastatin (LIPITOR) tablet 40 mg 40 mg, Oral, At bedtime, First dose on Thu 12/4/25 at 2100		2149	1414-MAR Hold 1504-MAR Unhold
cefTRIAxone (ROCEPHIN) 1 g in dextrose 5 % 50 mL IVPB ADD-EASE 1 g, Intravenous, at 100 mL/hr, Every 24 hours, First dose on Wed 12/3/25 at 2345, For 7 days, Indication: CAP Pneumonia		0000 0044- Stopped	0000 1414-MAR Hold 1504-MAR Unhold
doxycycline (VIBRAMYCIN) 100 mg in dextrose 5 % 100 mL IVPB ADD-EASE 100 mg, Intravenous, Administer over 60 Minutes, Every 12 hours, First dose on Wed 12/3/25 at 2345, For 7 days, Indication: CAP Pneumonia		0205 1201 2310	(1230) 1414-MAR Hold 1504-MAR Unhold

Continuous

Medication Order	12/03/2025	12/04/2025	12/05/2025
heparin 25000 units in d5w 250 mL (CANCELED) 12 Units/kg/hr × 76.6 kg (9.192 mL/hr, rounded to 9.2 mL/hr), Intravenous, Continuous, Starting on Wed 12/3/25 at 2145, Maximum initial rate: 1000 units/hr. *** BLACK BOX WARNING *** SOME PRODUCTS CONTAIN BENZYL ALCOHOL AS A PRESERVATIVE; USE OF THESE PRODUCTS IS CONTRAINDICATED IN NEONATES, Initial Dose (units/kg/hr): 12, Maximum Initial Dose (units/hr): 1000, Anti-Xa <0.1 Maintenance Dose Adjustment (units/kg/hr): 4, Anti-Xa <0.1 Bolus Dose (units/kg): 50, Anti-Xa <0.1 Additional Instructions: Rebolus at 50 units/kg (maximum rebolus dose: 4000 units). Then INCREASE current dose by 4 units/kg/hr., Anti-Xa 0.1-0.19 Maintenance Dose Adjustment (units/kg/hr): 3, Anti-Xa 0.1-0.19 Bolus Dose (units/kg): 25, Anti-Xa 0.1-0.19 Additional Instructions: Rebolus at 25 units/kg (maximum rebolus dose: 4000 units). Then INCREASE current dose by 3 units/kg/hr., Anti-Xa 0.2-0.29 Maintenance Dose Adjustment (units/kg/hr): 2, Anti-Xa 0.2-0.29 Bolus Dose (units/kg): 25, Anti-Xa 0.2-0.29 Additional Instructions: Rebolus at 25 units/kg (maximum rebolus dose: 4000 units). Then INCREASE current dose by 2 units/kg/hr., Anti-Xa less than or equal to 0.29 Maximum Rebolus Dose (units): 4000, Anti-Xa 0.3-0.7 Maintenance Dose	2219	0721- Handoff 0922 1618 1620 1905- Handoff	0056 0659- Handoff 1414-MAR Hold 1504-MAR Unhold

~~Adjustment (units/kg/hr): 0, Anti-Xa 0.3-0.7 Bolus Dose (units/kg): 0, Anti-Xa 0.3-0.7 Additional Instructions: Continue current infusion dose. No bolus needed. At therapeutic range., Anti-Xa 0.71-0.8 Maintenance Dose Adjustment (units/kg/hr): 1, Anti-Xa 0.71-0.8 Bolus Dose (units/kg): 0, Anti-Xa 0.71-0.8 Additional Instructions: DECREASING previous dose by 1 units/kg/hour., Anti-Xa 0.81-0.9 Maintenance Dose Adjustment (units/kg/hr): -2, Anti-Xa 0.81-0.9 Bolus Dose (units/kg): 0, Anti-Xa 0.81-0.9 Additional Instructions: DECREASING previous dose by 2 units/kg/hour., Anti-Xa 0.91-1 Maintenance Dose Adjustment (units/kg/hr): -3, Anti-Xa 0.91-1 Bolus Dose (units/kg): 0, Anti-Xa 0.91-1 Additional Instructions: Hold infusion for 1 hour. Then restart by DECREASING previous dose by 3 units/kg/hour., Anti-Xa >1 Maintenance Dose Adjustment (units/kg/hr): -4, Anti-Xa >1 Bolus Dose (units/kg): 0, Anti-Xa >1 Additional Instructions: Stop infusion and notify provider (Comment: repeat anti-Xa hourly until <0.7 then resume infusion at 4 units/kg/hr less than previously documented dose).~~

PRN

Medication Order	12/03/2025	12/04/2025	12/05/2025
acetaminophen (TYLENOL) tablet 650 mg 650 mg, Oral, Every 6 hours as needed, mild pain (1-3), moderate pain (4-6), headaches, Starting on Wed 12/3/25 at 2341, The max daily dose for patients less than 50 kg is 75 mg/kg/24 hours and for patients 50 kg or greater is 4000 mg/24 hours. Consider all source of Acetaminophen.			1414-MAR Hold 1504-MAR Unhold
artificial tears ophthalmic solution 1 drop 1 drop, Both Eyes, Every 1 hour as needed, dry eyes, Starting on Wed 12/3/25 at 2347		0039	1414-MAR Hold 1504-MAR Unhold
bisacodyl (DULCOLAX) suppository 10 mg 10 mg, Rectal, Daily as needed, constipation, Starting on Wed 12/3/25 at 2339, If milk of magnesia, bisacodyl tablet, docusate, senna or GLYCOLAX ineffective or not ordered			1414-MAR Hold 1504-MAR Unhold
docusate sodium (COLACE) tablet/capsule 100 mg 100 mg, Oral, Daily as needed, constipation, if milk of magnesia and bisacodyl tablet ineffective or not ordered, Starting on Wed 12/3/25 at 2339			1414-MAR Hold 1504-MAR Unhold
fentaNYL (SUBLIMAZE) injection (CANCELED) Code/trauma/sedation medication, Starting on Fri 12/5/25 at 1427, Intra-Procedure (Cath)			1427
hydrALAZINE (APRESOLINE) injection 10 mg 10 mg, Intravenous, Every 4 hours as needed, SBP > 170 or DBP > 100, Starting on Wed 12/3/25 at 2342			1414-MAR Hold 1504-MAR Unhold
iodixanol (VISIPAQUE) 270 MG/ML injection (CANCELED) Code/trauma/sedation medication, Starting on Fri 12/5/25 at 1443, Intra-Procedure (Cath)			1443
lidocaine 2 % injection (CANCELED) Code/trauma/sedation medication, Starting on Fri 12/5/25 at 1427, Intra-			1427 

Procedure (Cath)			
midazolam (VERSED) injection (CANCELED) Code/trauma/sedation medication, Starting on Fri 12/5/25 at 1427, Intra-Procedure (Cath)			1427
nitroglycerin (NITROSTAT) SL tablet 0.4 mg 0.4 mg, Sublingual, Every 5 minutes x3 as needed, chest pain, Starting on Wed 12/3/25 at 2342, May administer up to 3 doses per episode * * * DO NOT CRUSH OR CHEW * * *			1414-MAR Hold 1504-MAR Unhold
nitroglycerin (TRIDIL) injection (CANCELED) Code/trauma/sedation medication, Starting on Fri 12/5/25 at 1429, Intra-Procedure (Cath)			1429
ondansetron (ZOFTRAN) injection 4 mg 4 mg, Intravenous, Every 4 hours as needed, nausea, vomiting, Starting on Wed 12/3/25 at 2339, If patient is NPO or unable to tolerate PO			1414-MAR Hold 1504-MAR Unhold
ondansetron (ZOFTRAN-ODT) disintegrating tablet 4 mg 4 mg, Oral, Every 6 hours as needed, nausea, vomiting, Starting on Wed 12/3/25 at 2339			1414-MAR Hold 1504-MAR Unhold
polyethylene glycol (GLYCOLAX) powder 17 g 17 g, Oral, Daily as needed, constipation, constipation if milk of magnesia, bisacodyl tablet, docusate sodium and senna ineffective or not ordered, Starting on Wed 12/3/25 at 2339			1414-MAR Hold 1504-MAR Unhold
sodium chloride (NS) 0.9 % flush 3 mL 3 mL, Intravenous, Every 8 hours as needed, line care, Starting on Fri 12/5/25 at 1503			
verapamil (ISOPTIN) injection (CANCELED) Code/trauma/sedation medication, Starting on Fri 12/5/25 at 1429, Intra-Procedure (Cath)			1429

Orders - documented in this encounter

Medications Ordered That Might Not Have Been Administered	Count	Last Ordered Date	First Ordered Date
fentaNYL (SUBLIMAZE) injection	1	12/05/2025	
iodixanol (VISIPAQUE) 270 MG/ML injection	1	12/05/2025	
lidocaine 2 % injection	1	12/05/2025	
midazolam (VERSED) injection	1	12/05/2025	
nitroglycerin (TRIDIL) injection	1	12/05/2025	
sodium chloride (NS) 0.9 % flush 3 mL	1	12/05/2025	
verapamil (ISOPTIN) injection	1	12/05/2025	
acetaminophen (TYLENOL) tablet 650 mg	1	12/03/2025	
bisacodyl (DULCOLAX) suppository 10 mg	1	12/03/2025	
docusate sodium (COLACE) tablet/capsule 100 mg	1	12/03/2025	
heparin (porcine) injection 1,900 Units	2	12/03/2025	
heparin (porcine) injection 3,800 Units	1	12/03/2025	
hydrALAZINE (APRESOLINE) injection 10 mg	1	12/03/2025	
nitroglycerin (NITROSTAT) SL tablet 0.4 mg	1	12/03/2025	
ondansetron (ZOFTRAN) injection 4 mg	1	12/03/2025	
ondansetron (ZOFTRAN-ODT) disintegrating tablet 4 mg	1	12/03/2025	
PHARMACY TO DOSE HEPARIN 1 Consult	1	12/03/2025	
polyethylene glycol (GLYCOLAX) powder 17 g	1	12/03/2025	
Nursing	Count	Last Ordered Date	First Ordered Date
DISCHARGE INSTRUCTIONS	1	12/05/2025	
Code Status	Count	Last Ordered Date	First Ordered Date
FULL CODE	1	12/05/2025	
Admission	Count	Last Ordered Date	First Ordered Date
ADMIT TO INPATIENT	1	12/03/2025	
Transfer	Count	Last Ordered Date	First Ordered Date
IP BED REQUEST	1	12/03/2025	
Discharge	Count	Last Ordered Date	First Ordered Date
DISCHARGE PATIENT	1	12/05/2025	
CORE MEASURES	Count	Last Ordered Date	First Ordered Date
REASON FOR NO VTE PROPHYLAXIS - HOSPITAL ADMISSION - MEDICATIONS	1	12/03/2025	
Case Request	Count	Last Ordered Date	First Ordered Date
CASE REQUEST CATH LAB	1	12/05/2025	

Additional Health Concerns - documented as of this encounter

Infection	Onset Date	Last Indicated	Resolved Time
Rule-out COVID-19	12/03/2025	12/04/2025	12/04/2025 6:29 AM PST

Care Teams - documented as of this encounter

Team Member	Relationship	Specialty	Start Date	End Date
Maples, Sarah NPI: 1598182479 1664 N VIRGINIA ST BRIGHAM BLDG 316UNSOM RENO, NV 89557-0001 775-784-1533 (Work)	PCP - General		12/1/25	

Patient Demographics

Patient Address	Patient Name	Communication
222 GORDON LN (Home) DAYTON, NV 89403	Michelle Caruso	909-744-2347 (Mobile) FROM.MICHELLE@YAHOO.COM
Former (Feb. 17, 2025 - Mar. 03, 2025): (Home)		
Language	Race / Ethnicity	Marital Status
English - Written (Preferred)	White / Not Hispanic or Latino	Single

Patient Contacts

Contact Name	Contact Address	Communication	Relationship to Patient
Sebastian Lopez	Unknown	808-278-8802 (Mobile)	Son, Emergency Contact

Document Information

Primary Care Provider	Other Service Providers	Document Coverage Dates
Sarah Maples (Dec. 01, 2025 - Present) NPI: 1598182479 775-784-1533 (Work) 1664 N VIRGINIA ST BRIGHAM BLDG 316UNSOM RENO, NV 89557-0001 PHS NEVADA SERVICE AREA 702-649-7711 (Work) 1409 E Lake Mead Blvd NORTH LAS VEGAS, NV 89030		Dec. 03, 2025 - Dec. 05, 2025
Custodian Organization		
PHS NEVADA SERVICE AREA 702-649-7711 (Work) 1409 E Lake Mead Blvd NORTH LAS VEGAS, NV 89030		
Encounter Providers		Encounter Date
Daniel B Van Bibber, MD (Attending) NPI: 1407112048 775-770-3188 (Work) 775-770-3588 (Fax) 235 West Sixth Street, ER Dept. Reno, NV 89503 Emergency Medicine	John E Stewart, MD (Attending) NPI: 1083715007 775-770-6490 (Work) 775-770-3944 (Fax) 411 West Sixth Street Reno, NV 89503 Hospitalist	Dec. 03, 2025 - Dec. 05, 2025
Manvi R Nagam, MD (Attending) NPI: 1811482938 775-770-6490 (Work) 775-770-3944 (Fax) 411 West Sixth Street Reno, NV 89503 Hospitalist	Deepthi Ciddi, MD (Admitting) NPI: 1932620986 775-770-6490 (Work) 775-770-3944 (Fax) 411 West Sixth Street Reno, NV 89503 Hospitalist	
Ladrina Allen, RN (Attending) Emergency Medicine	Saul Lachino, RN (Attending) Emergency Medicine	
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Ingrid Segura, CNA (Attending)	Lois Fairman, RN (Attending)	